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Roche

HY 2026 results

Basel, 23 July 2026



Group

Thomas Schinecker
Chief Executive Officer

Performance

Portfolio progress

Outlook

HY 2026: COP growth at +10% ahead of sales growth at +6%*

HY results all at CER

- Group sales growth +6%, Pharma +6% and Diagnostics +3% (still impacted by the healthcare pricing reforms in China)
- Strong bottom line performance with Core OP +10% and Core OP margin +1.7%p; Core EPS +9%
- LOE impact of roughly CHF 0.6bn expected for 2026

Key milestones achieved in Q2

- Pharma regulatory: US approval Tecentriq in ctDNA-pos high-risk MIBC, US & EU filing Tecentriq in stage III dMMR/MSI-H colon cancer, US filing Enspryng in TED, US & EU filing Gazyva in MN, EU filing Gazyva in INS
- Pharma readouts: Positive Ph III (KRASCENDO 1) divarasib in 2L+ KRAS G12C+ NSCLC, positive Ph III (POYANG) Vabysmo in mCNV
- Business development: PathAI** acquisition to transform AI-driven diagnostics, Nurix Therapeutics collaboration for bexobrutideg (BTK degrader)
- Diagnostics launches: AXELIOS 1 next generation sequencing platform, Elecsys pTau217 CE mark, Elecsys IGRA TB CE mark, cobas HDV CE mark

Significant newsflow in 2026 ahead

- Pivotal Ph III readouts: Itovebi in 1L *PIK3CA*-mut HER2+ mBC and post CDKi HR+/HER2- mBC, Lunsumio in 2L+ FL, sefaxersen in IgAN
- Ph II readouts: emugrobart in obesity, enicepatide (CT-388) in obesity with T2D (study 104), CT-996 in obesity
- Diagnostic launches: VENTANA BAP1, eplex gastrointestinal panel

*All at CER; **pending deal closure; BAP1: BRCA1 associated protein-1; BTK: Bruton tyrosine kinase; CER: Constant exchange rates (avg. full year 2025); CDK: Cyclin-dependent kinase; dMMR: Deficient DNA mismatch repair; FL: Follicular lymphoma; HDV: Hepatitis D virus; HER2: Human epidermal growth factor; HR: Hormone receptor; IgAN: IgA nephropathy; INS: Idiopathic nephrotic syndrome; LOE: Loss of exclusivity incl. global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis, Actemra; (m)BC: (Metastatic) breast cancer; mCNV: Myopic choroidal neovascularization; MIBC: Muscle-invasive bladder cancer; MN: Membranous nephropathy; MSI-H: Microsatellite instability-high; NSCLC: Non-small cell lung cancer; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; T2D: Type-2 diabetes; TB: Tuberculosis; TED: Thyroid eye disease

HY 2026: Strong Pharma sales driving Group growth

Sales impacted by low flu season and Diagnostics healthcare pricing reforms in China

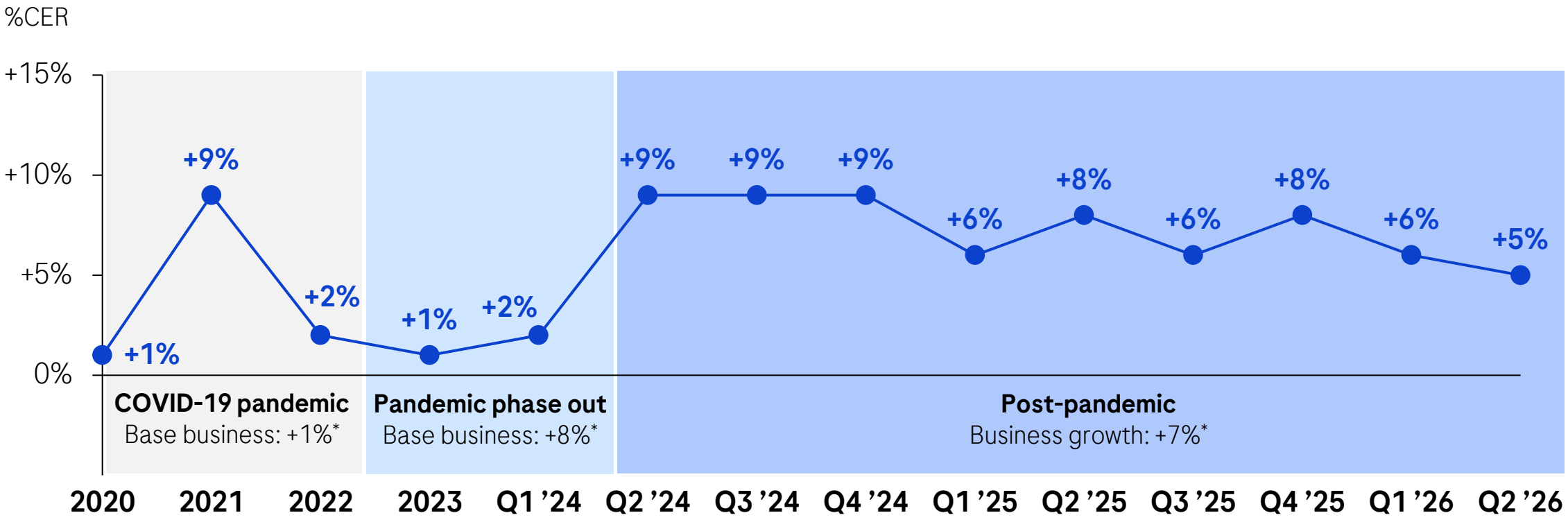
	HY 2026 CHFbn	HY 2025 CHFbn	Change in % CHF	Change in % CER
Pharmaceuticals Division	**23.6**	24.0	**-1**	**6**
Diagnostics Division	**6.7**	7.0	**-3**	**3**
Roche Group	**30.4**	30.9	**-2**	**6**

CER: Constant exchange rates (avg. full year 2025)

HY 2026: Consistent strong growth

Sales impacted by low flu season and Diagnostics healthcare pricing reforms in China

Group sales growth



*Average growth rate of quarterly CER growth rates for specified period; All growth rates at CER: Constant exchange rates (avg. full year of respective years); Base business: Pharma excluding Ronapreve and Diagnostics excluding COVID-19 diagnostic tests

Key growth drivers of the Roche portfolio

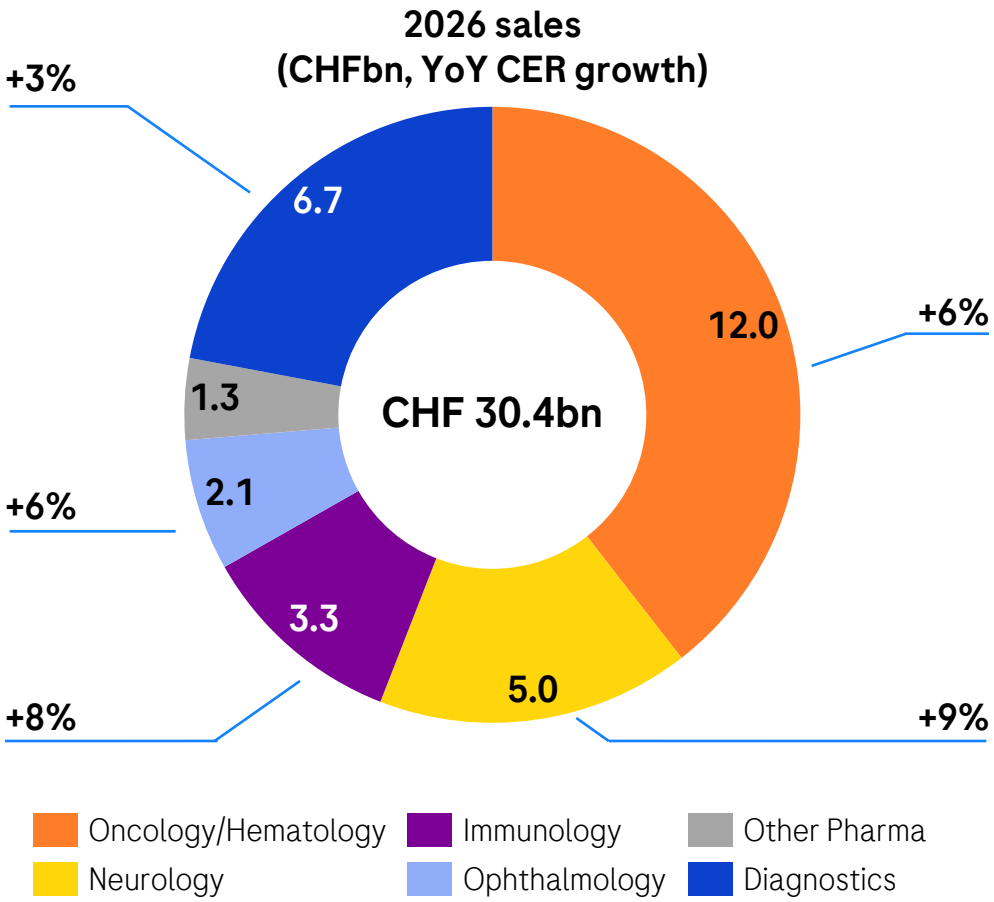
Growth driven by a broad and diversified portfolio

Diagnostics: Pathology Lab with strong growth; Core Lab growing despite China healthcare pricing reforms

Vabysmo: Continued global growth and market share gains

Xolair: Continued strong growth in food allergy, first biosimilar expected H2 '26

Gazyva: Strong initial uptake in LN



Phesgo: Conversion ongoing; HER2+ BC franchise to peak in 2026 with strong tail

Tecentriq: Accelerated growth driven by new indications

Alecensa: Flat growth expected for FY

Polivy: Continued strong 1L DLBCL uptake

Columvi: Driven by 2L+ DLBCL

Lunsumio: Strong 3L+ FL uptake

Hemlibra: Continued strong global growth

Ocrevus: Global growth driven by SC

Evrysdi: Expanding strong position in SMA

Totals and subtotals may include differences due to rounding; BC: Breast cancer; CER: Constant exchange rates (avg. full year 2025); DLBCL: Diffuse large B-Cell lymphoma; FL: Follicular lymphoma; HER2: Human epidermal growth factor; LN: Lupus nephritis; SC: Subcutaneous; SMA: Spinal muscular atrophy

Performance

Portfolio progress

Outlook

Pharma: Continuous pipeline progress through partnering and M&A

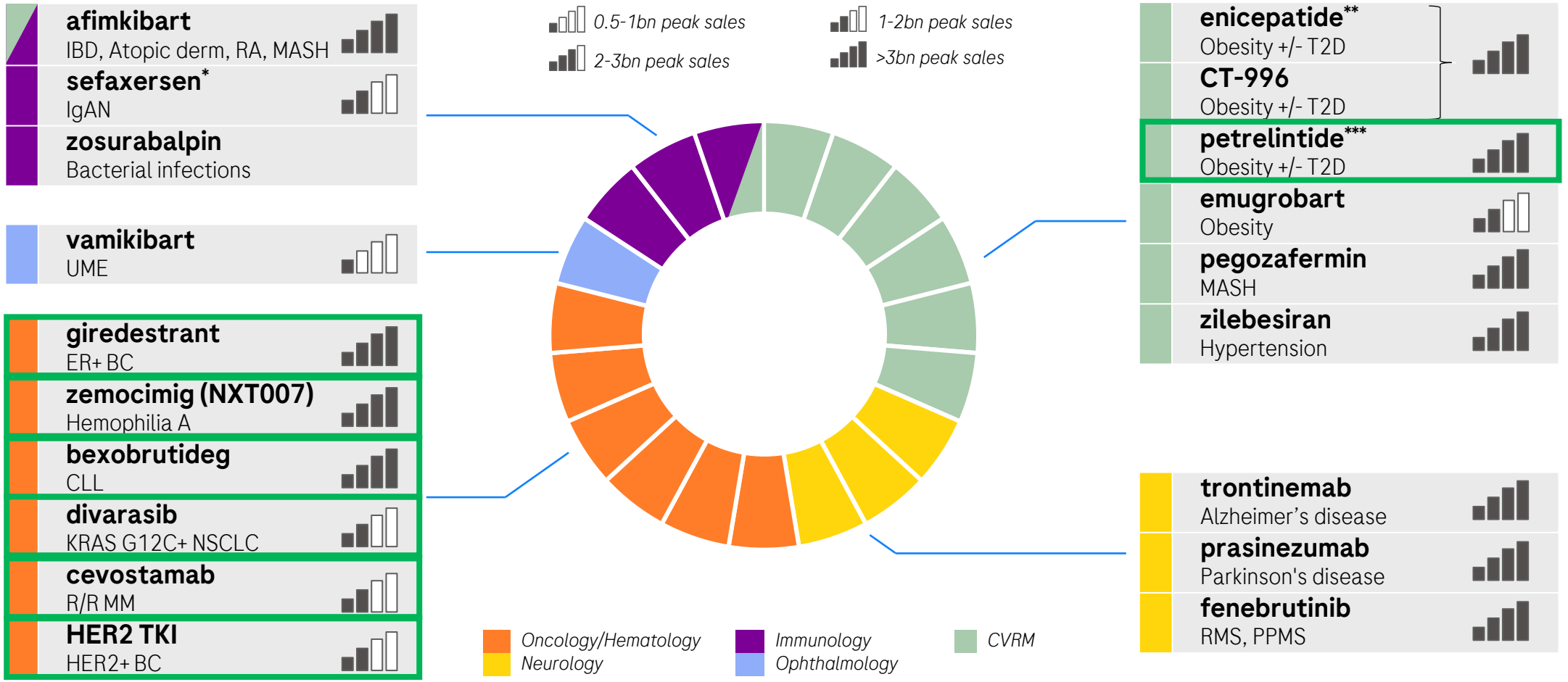
Addition of a BTK degrader with BIC potential through Nurix deal to strengthen oncology and beyond

ANTLERA	赞荣医药 Zion Pharma	Regor Therapeutics	NURIX	Telavant	Alnylam PHARMACEUTICALS	CARMOT THERAPEUTICS	ZEALAND PHARMA	89bio
Ophthalmology	**Onc/Heme**	**Onc/Heme**	**Onc/Heme, Immuno & Neuro**	**Immunology**	**CVRM**	**CVRM**	**CVRM**	**CVRM**
Wnt agonist in retinopathies	**HER2 TKI** in breast cancer	**CDKi portfolio** in breast cancer	**BTK degrader** in CLL, immunology & neurology	**Anti-TL1A** in inflammatory bowel disease	**siRNA targeting AGT** in hypertension	**Dual GLP-1/GIP RA** in obesity +/- diabetes	**Long-acting amylin analog** in obesity	**FGF21 analog** in MASH
Pre-clinical	**Ph II**	**Ph I**	**Ph III**	**Ph III**	**Ph III**	**Ph III**	**Ph II**	**Ph III**

ADC: Antibody-drug conjugate; AGT: Angiotensinogen; BIC: Best in class; BTK: Bruton tyrosine kinase; CDKi: Cyclin dependent kinase inhibitor; CLL: Chronic lymphocytic leukemia; CVRM: Cardiovascular, renal & metabolism; FGF21: Fibroblast growth factor 21; GLP-1/GIP: Glucagon-like peptide-1 / glucose insulinotropic polypeptide receptor agonist; MASH: Metabolic dysfunction-associated steatohepatitis; siRNA: Small interfering RNA; TKI: Tyrosine kinase inhibitor; TL1A: Tumor necrosis factor-like cytokine 1A; Wnt: Wingless-related integration site

Pharma: Up to 19 NMEs with launch potential by 2030

Positive Ph III (KRASCENDO 1) divarasib in 2L+ KRAS G12C+ NSCLC; upcoming sefaxersen readout in IgAN



*in collaboration with Ionis; **CT-388; ***in collaboration with Zealand Pharma; Peak sales shown unadjusted; CVRM: Cardiovascular, renal and metabolism; IgAN: IgA nephropathy; NME: New molecular entity; NSCLC: Non-small cell carcinoma

Diagnostics: Key Business Development across Customer Areas

PathAI acquisition* to expand pathology portfolio; recent deals to strengthen oncology ecosystem

STRATOS genomics inc.	TIB MOLBIOL	GenMark Dx	Freenome	SAGA DIAGNOSTICS	PathAI *	lumiraDx™
Molecular Lab	Molecular Lab	Molecular Lab	Molecular Lab	Molecular Lab	Pathology Lab	Near Patient Care
Sequencing-by-expansion technology	Ultra-rapid PCR assays development	Syndromic infectious disease testing	Early detection cancer screening	Ultra sensitive MRD technology	AI-powered pathology solutions	Portable PoC testing
2020	2021	2021	2021/2025	2026	2026	2024

Recent deals to expand oncology end-to-end offering

from early detection through diagnosis to disease monitoring

*pending deal closure; MRD: Minimal residual disease; PoC: Point of care

Diagnostics: Key technology launches driving future growth

AXELIOS 1 sequencing solution launched in June 2026

Key platform launches

Near Patient Care

Accu-Chek SmartGuide



Launched in 2024

Core Lab

cobas i 601 Mass Spec



Launched in CE markets in 2024, full US launch in 2026

Molecular Lab

AXELIOS 1 (SBX)



Launched in June 2026

Selected assays

- **cobas Lp(a) Gen 2** — Cardiovascular risk
- **Elecsys hs-TnT Gen 6** — Acute coronary syndrome
- **cobas HPV genotyping**[1] — Cervical cancer
- **FoundationOne Monitor**[2] — ctDNA monitoring
- **Elecsys pTau217** — Alzheimer's disease
- **Elecsys NfL** — Multiple sclerosis
- **cobas BV/CV** — BV/CV
- **Elecsys IGRA TB** — Tuberculosis

Customer Areas: Core Lab; Molecular Lab; Near Patient Care; Pathology Lab

Therapeutic Areas: *Cardiometabolic*; *Oncology*; *Neurology*; *Infectious Diseases*

1. In development; 2. Laboratory developed test which has not been reviewed or approved by the FDA; BV/CV: Bacterial vaginosis, candida vaginitis; HPV: Human papillomavirus; hs-TnT: high sensitivity Troponin T; IGRA: Interferon-gamma release assay; NfL: Neurofilament light chain; Lp(a): Lipoprotein(a); TB: Tuberculosis

Performance

Portfolio progress

Outlook

2026 guidance confirmed

LOE impact of roughly CHF 0.6bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra
Note: Guidance according to the official publication in the respective press release of Roche Holding Ltd



Finance

Alan Hippe
Chief Financial Officer

Results

Cash & balance sheet

Currency guidance & outlook

HY 2026: Group performance

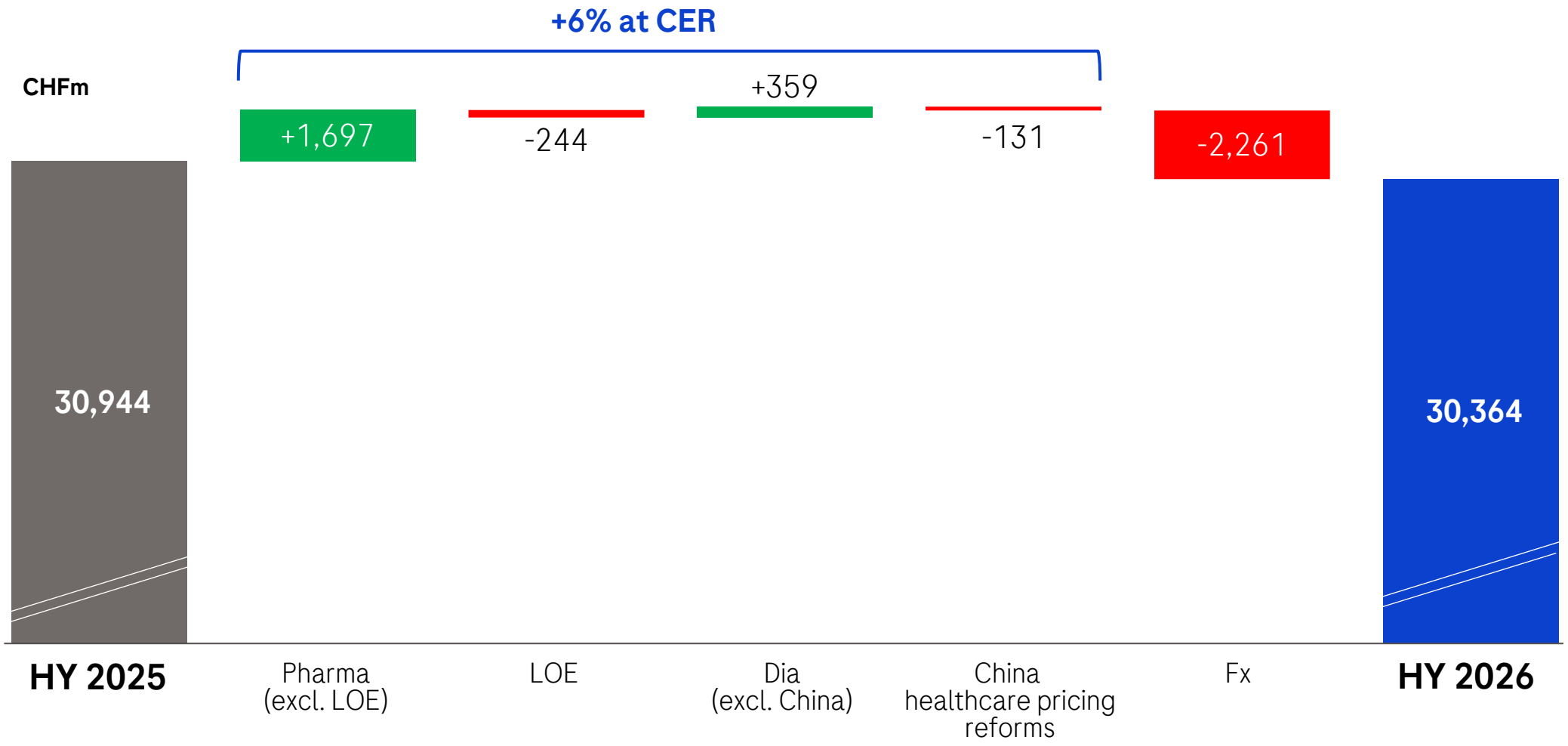
Sales increase of +6% and Core EPS increase of +9% at CER

	2026 CHFm	2025 CHFm	Change in % CHF	Change in % CER
Sales	**30,364**	**30,944**	**-2**	**+6**
Core operating profit	**11,856**	**12,010**	**-1**	**+10**
as % of sales	*39.0*	*38.8*		
Core net income	**9,216**	**9,319**	**-1**	**+11**
as % of sales	*30.4*	*30.1*		
Core EPS (in CHF)	**10.85**	**11.08**	**-2**	**+9**
IFRS net income	**7,325**	**7,832**	**-6**	**+6**
as % of sales	*24.1*	*25.3*		
Operating free cash flow	**6,264**	**6,114**	**+2**	**+21**
as % of sales	*20.6*	*19.8*		
Free cash flow	**4,197**	**3,319**	**+26**	**+58**
as % of sales	*13.8*	*10.7*		

All numbers in CHFm, except Core EPS in CHF; CER: Constant exchange rates (avg. full year 2025); EPS: Earnings per share

HY 2026: Group sales growth +6%

Business growth exceeding impact of healthcare pricing reforms in China and LOE



Totals may include differences due to rounding; CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra

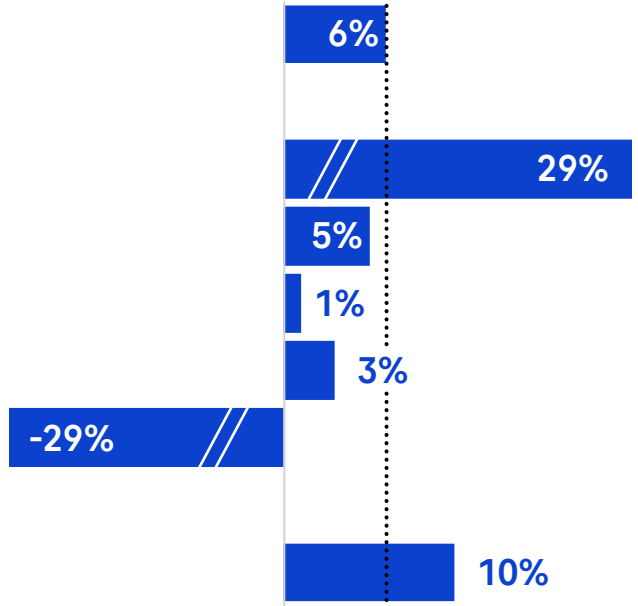
HY 2026: Group core operating profit

Core operating profit up +10%, driven by higher sales and effective cost management

	2026	
	CHFm	Abs. CERm
Sales	**30,364**	**+1,681**
Other revenue	1,088	+258
Cost of sales	-7,673	-391
R&D	-5,765	-66
SG&A	-6,370	-221
OOI&E	212	-87
Core operating profit	**11,856**	**+1,173**
Core OP as % of sales	*39.0%*	
At CER		*39.9%*
		(HY 2025: 38.2%)

2026 vs. 2025

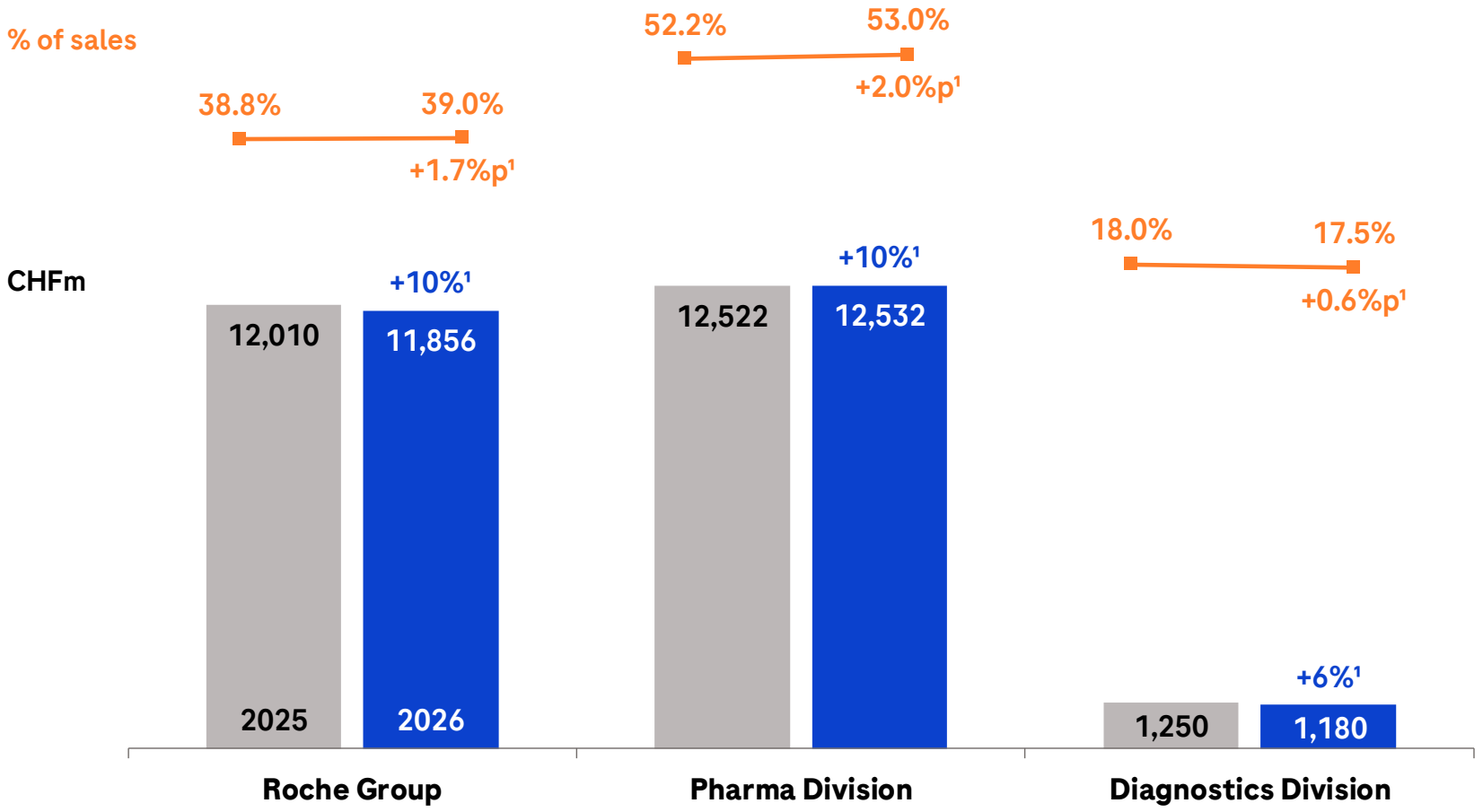
CER growth



-1% in CHF

CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration; Totals may include differences due to rounding

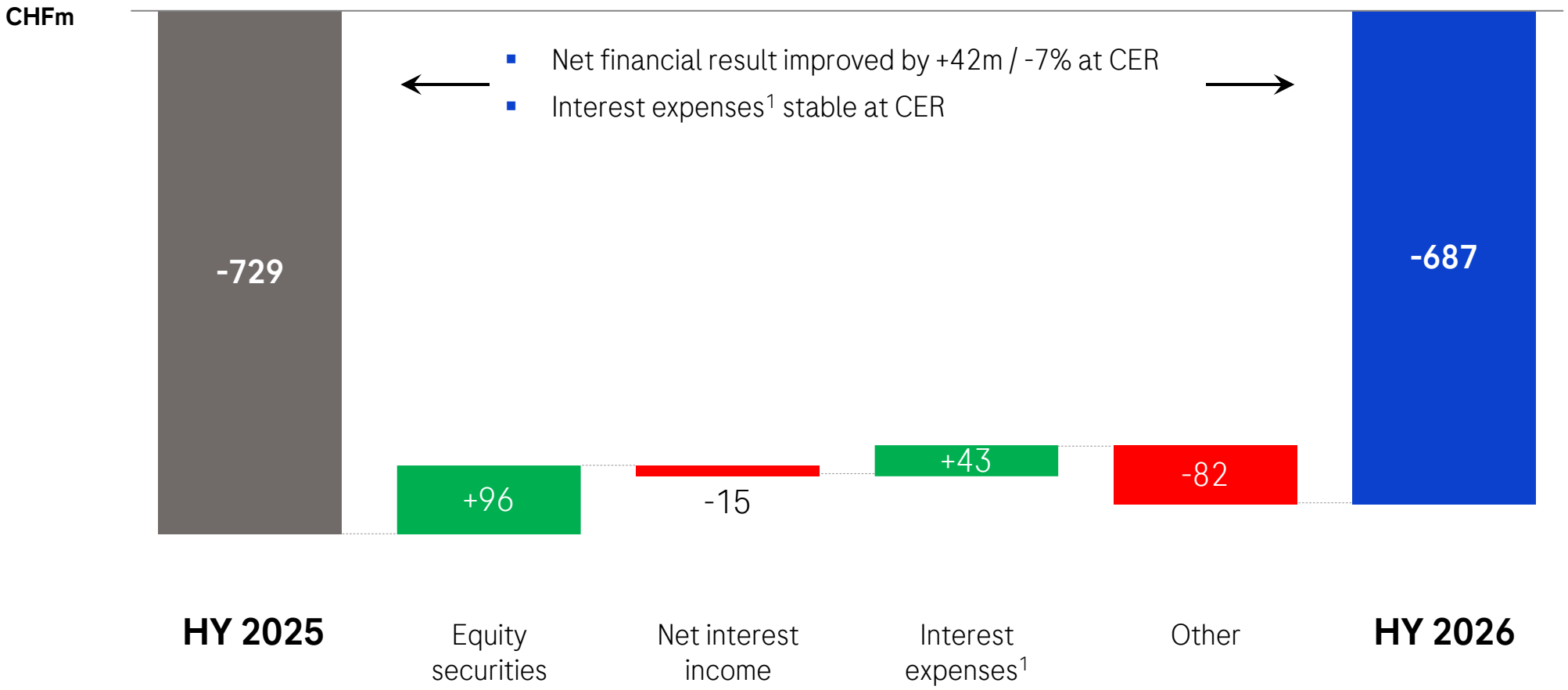
HY 2026: Core operating profit and margin



Note: Group core operating profit includes -1.9bn from Corporate (-1.8bn in 2025); 1. At CER: Constant exchange rates (avg. full year 2025)

HY 2026: Core net financial result

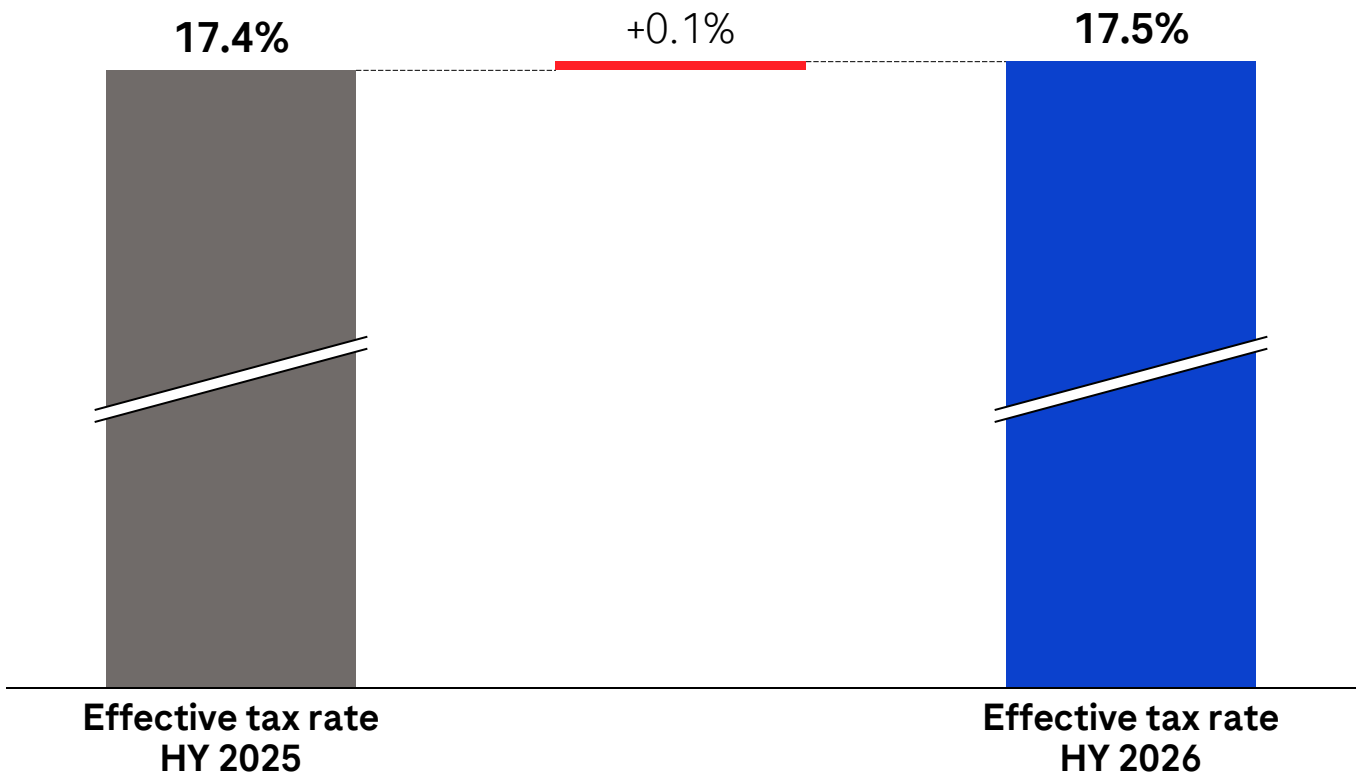
Improvement mainly driven by higher gains on equity securities



1. Incl amortization of debt discount and net gains on interest rate derivatives; CER: Constant exchange rates (avg. full year 2025)

HY 2026: Core tax rate

Tax rate stable compared to HY 2025



All at reported exchange rates

HY 2026: Core EPS

Increase in operations partially offset by lower gains on product disposals



All at CER: Constant exchange rates (avg. full year 2025); 1. Core operating profit excl. gains/losses on product disposals and gains from disposal of subsidiaries (InterMune sale)

HY 2026: Non-core and IFRS income

Non-core items up vs. PY mainly due to higher impairment of intangible assets

	2026 CHFm	2025 CHFm	Var. at CERm	Change in % CHF	Change in % CER
Core operating profit	**11,856**	**12,010**	**+1,173**	**-1**	**+10**
Global restructuring plans	-765	-1,023	+222		
Amortisation of intangible assets	-391	-348	-67		
Impairment of intangible assets[1]	-1,045	-235	-855		
M&A and alliance transactions	18	10	+8		
Legal & environmental[2]	-2	-84	+82		
Total non-core operating items	-2,185	-1,680	-609		
IFRS operating profit	**9,671**	**10,330**	**+564**	**-6**	**+6**
Total financial result & taxes	-2,346	-2,498	-77		
IFRS net income	**7,325**	**7,832**	**+488**	**-6**	**+6**

Totals & sub-totals may include differences due to rounding; 1. Incl. goodwill; 2. Incl. pension plan settlements; CER: Constant exchange rates (avg. full year 2025)

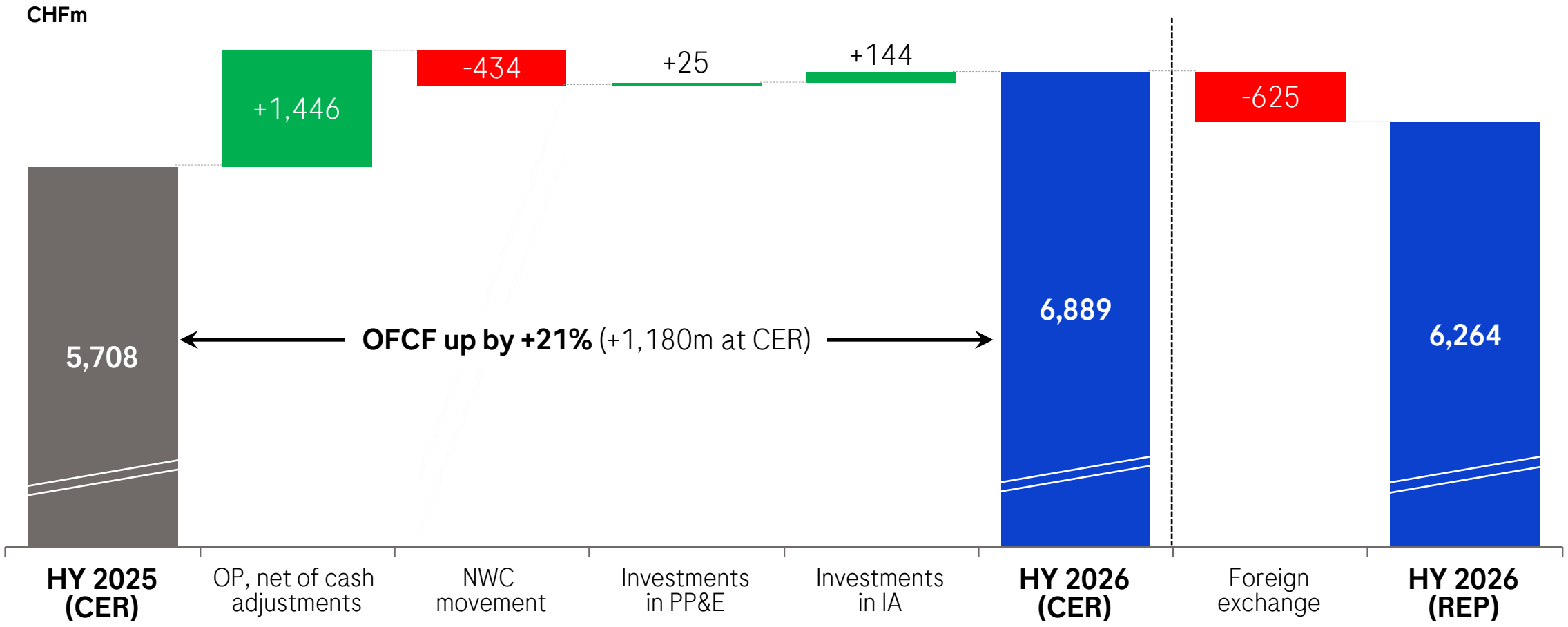
Results

Cash & balance sheet

Currency guidance & outlook

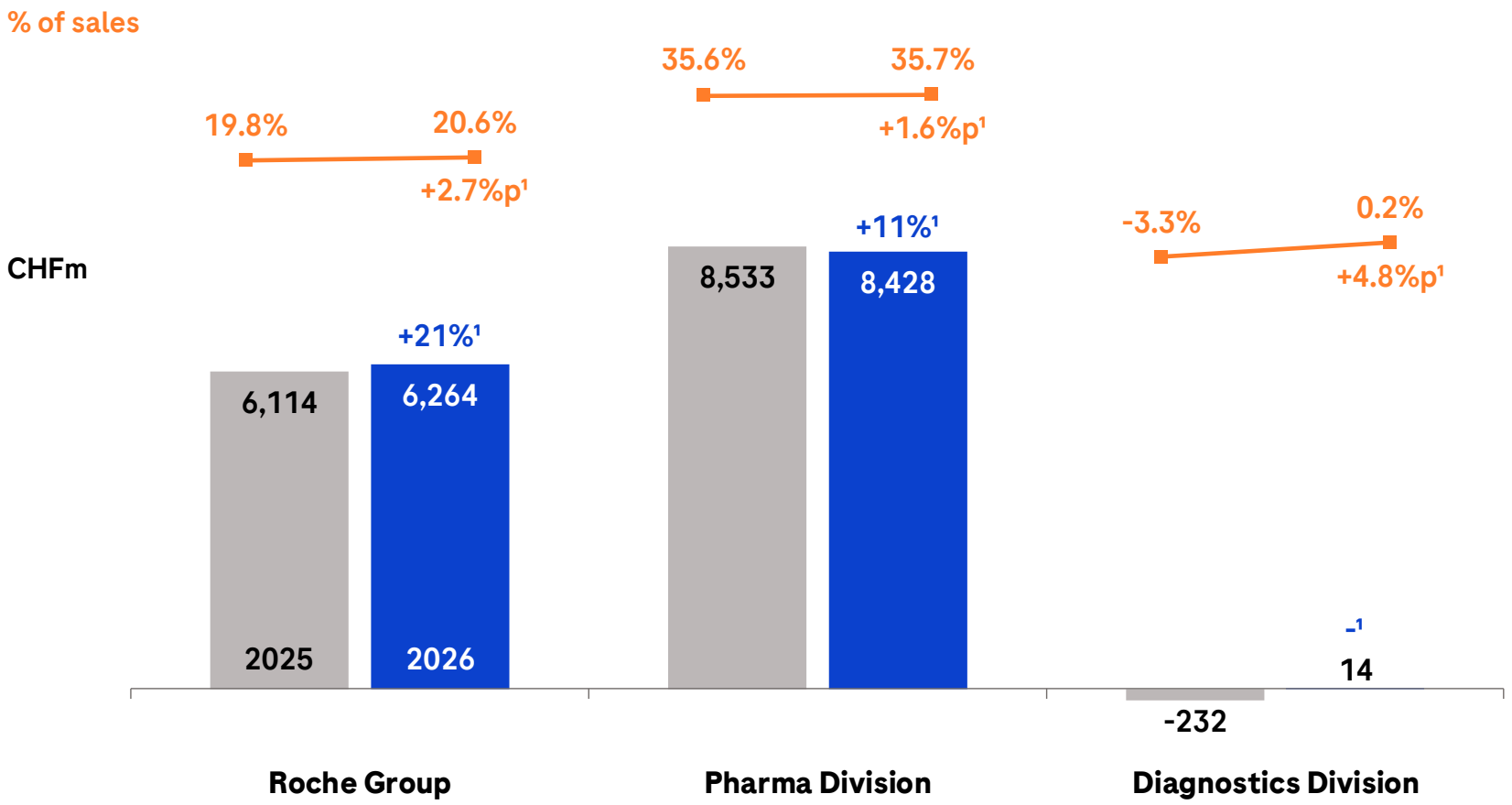
HY 2026: Group operating free cash flow

OFCF up by +21% mainly driven by higher cash operating profit, partly offset by NWC movement



CER: Constant exchange rates (avg. full year 2025); IA: Intangible assets; NWC: Net working capital; OP: Operating profit; PP&E: Property, plant & equipment incl. lease liability paid; Totals may include differences due to rounding

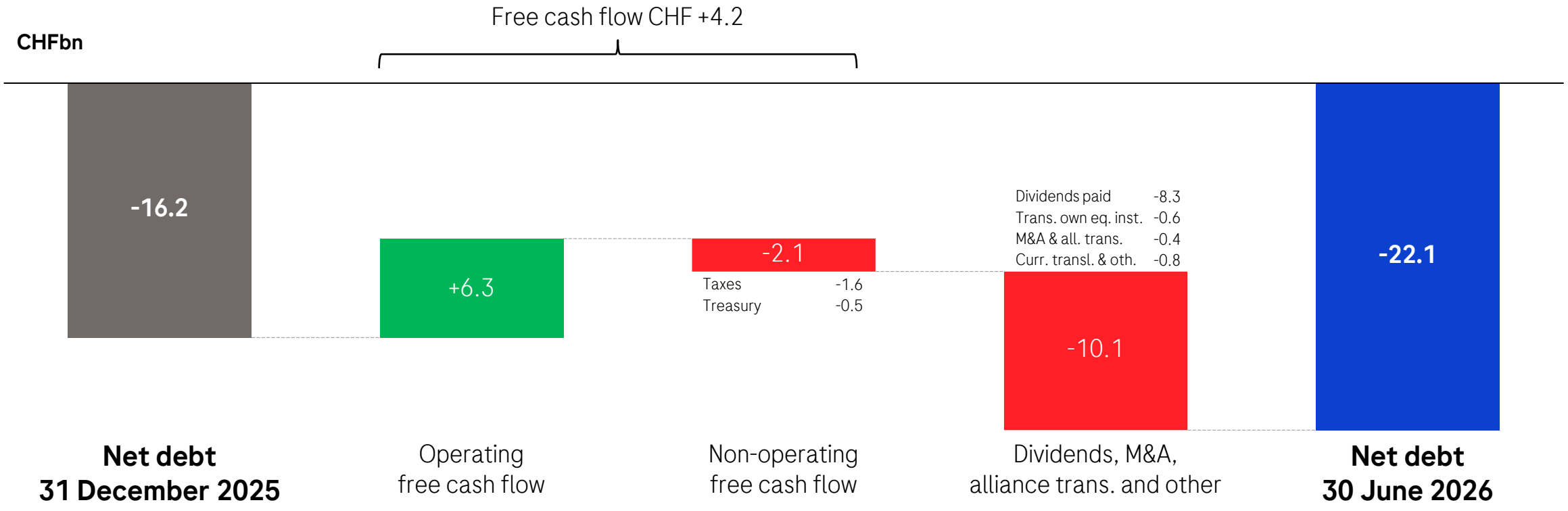
HY 2026: Operating free cash flow and margin



Note: Group operating free cash flow includes -2.2bn from Corporate (-2.2bn in 2025); 1. At CER: Constant exchange rates

HY 2026: Group net debt development

Net debt higher by CHF -5.9bn vs. YE 2025

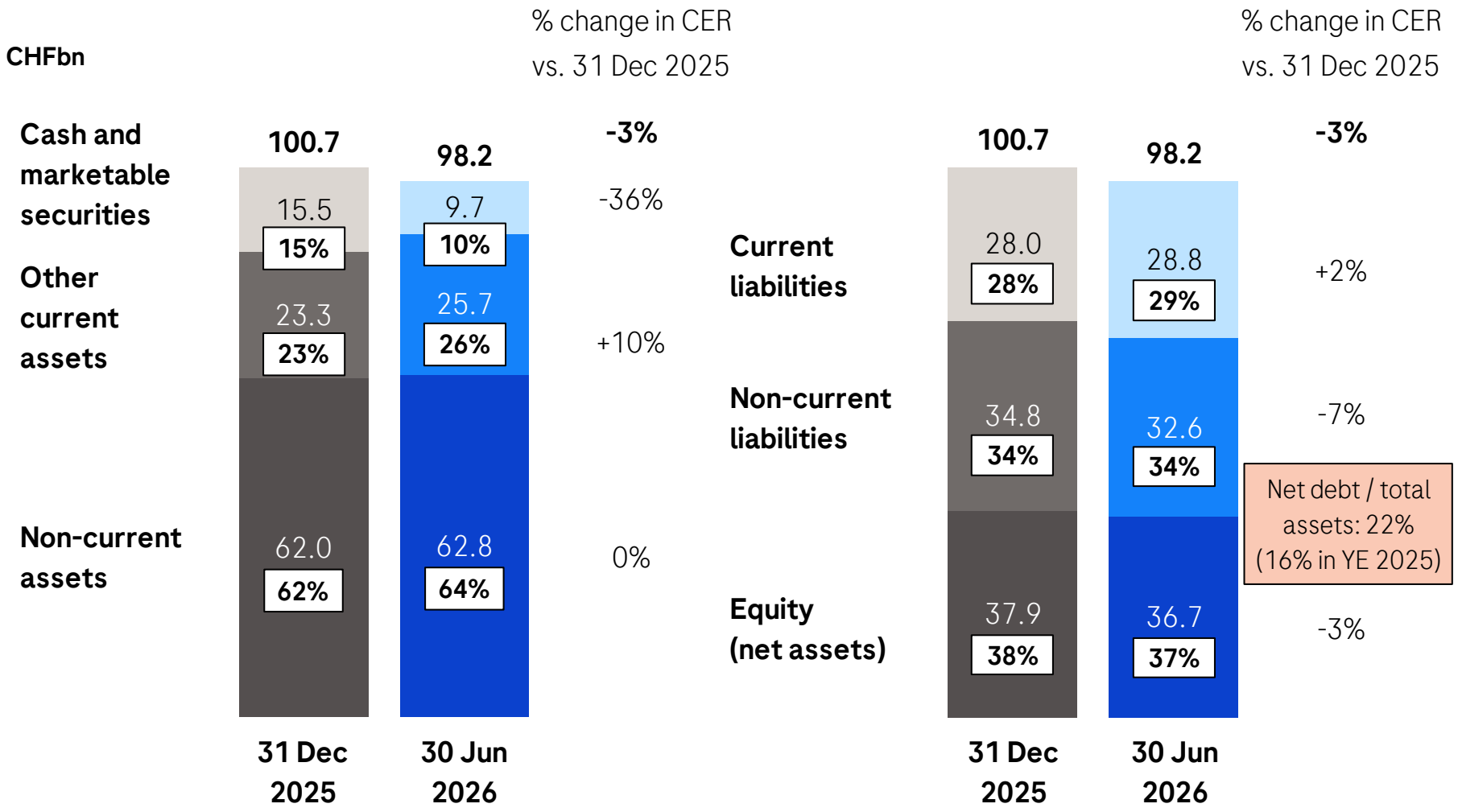


		Intangible assets	Equity	M&A	Total
Thereof investments in innovation:	**2026**	-1.3	-0.1	-0.4	**-1.8**
	2025	-1.5	0.0	-0.9	**-2.4**

M&A: Mergers & acquisitions

Balance sheet 30 June 2026

Equity ratio at 37% (31 Dec 2025: 38%; 30 Jun 2025: 35%)



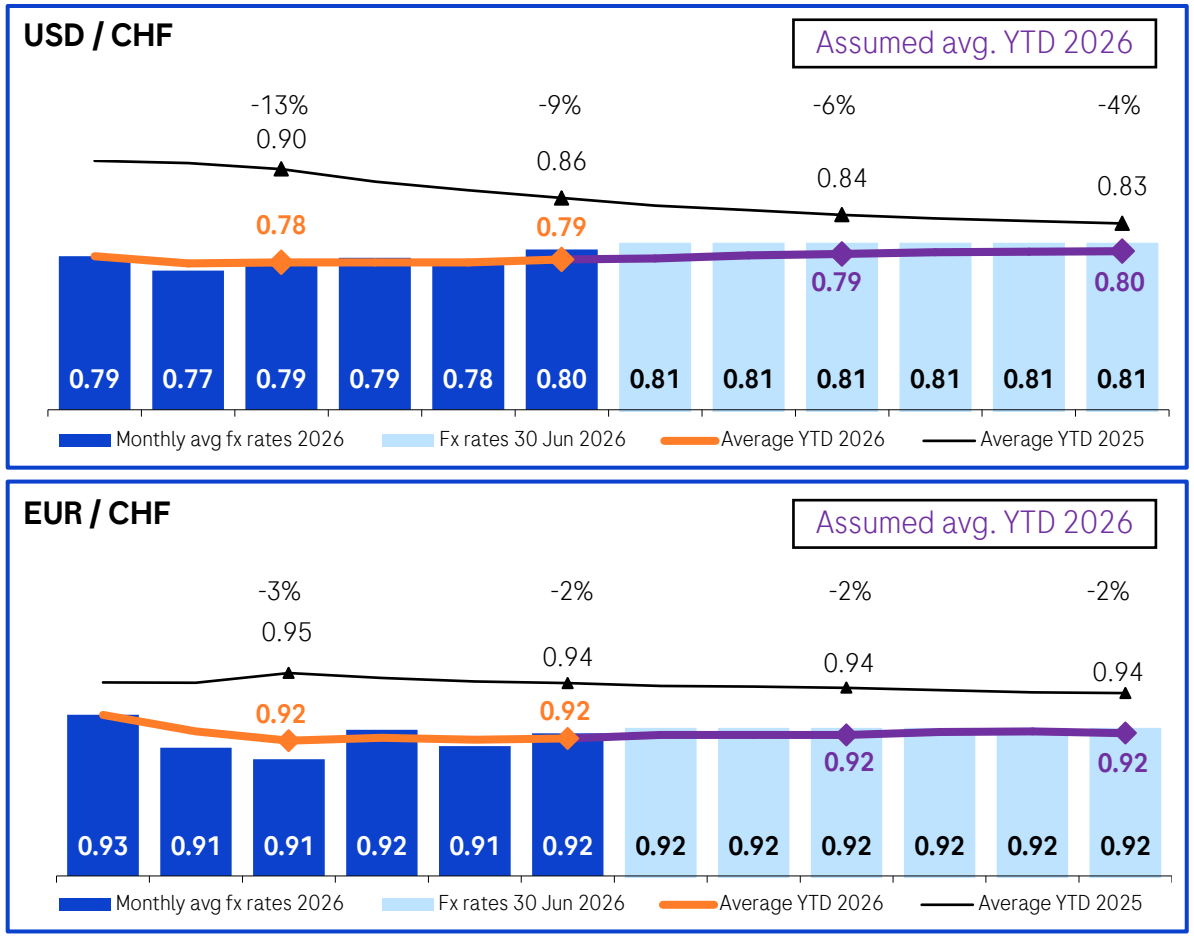
CER: Constant exchange rates (avg. full year 2025)

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Expected 2026 currency impact



Assuming the 30 June 2026 exchange rates remain stable until the end of 2026.

2026 impact[1] is expected to be (%p):

	Q1	Q2	Q3	Q4
Sales	**-11**	**-4**	0	0

	Q1	HY	YTD Sep	FY
Sales	**-11**	**-8**	-5	-4
Core operating profit		**-11**		-6
Core EPS		**-11**		-6

1. On Group growth rates

2026 guidance confirmed

LOE impact of roughly CHF 0.6bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra
Note: Guidance according to the official publication in the respective press release of Roche Holding Ltd



Pharmaceuticals Division

Teresa Graham
CEO Roche Pharmaceuticals

HY 2026: Pharmaceuticals sales

All regions delivering growth

	HY 2026 CHFm	HY 2025 CHFm	Change in % CHF	Change in % CER
Pharmaceuticals Division	**23,629**	**23,985**	**-1**	**6**
United States	12,229	12,670	-3	6
Europe	4,520	4,566	-1	1
Japan	1,360	1,425	-5	11
International	5,520	5,324	4	10

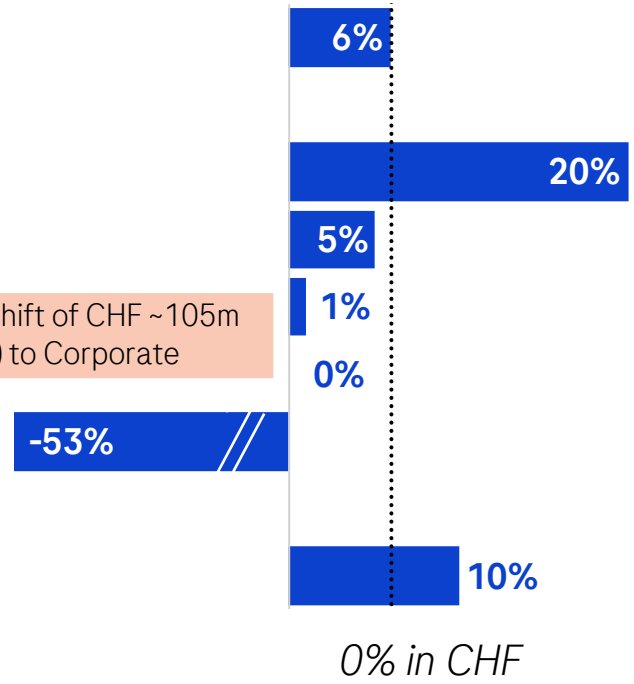
CER: Constant exchange rates (avg. full year 2025)

HY 2026: Pharmaceuticals core operating profit

Core operating profit growing ahead of sales with +10%, driven by effective cost management

	2026	
	CHFm	Abs. CERm
Sales	**23,629**	**+1,453**
Other revenue	965	+168
Cost of sales	-4,231	-217
R&D	-4,895	-40
SG&A	-3,031	-10
OOI&E	95	-109
Core operating profit	**12,532**	**+1,244**
Core OP as % of sales	*53.0%*	
At CER		*53.8%*
		(HY 2025: 51.8%)

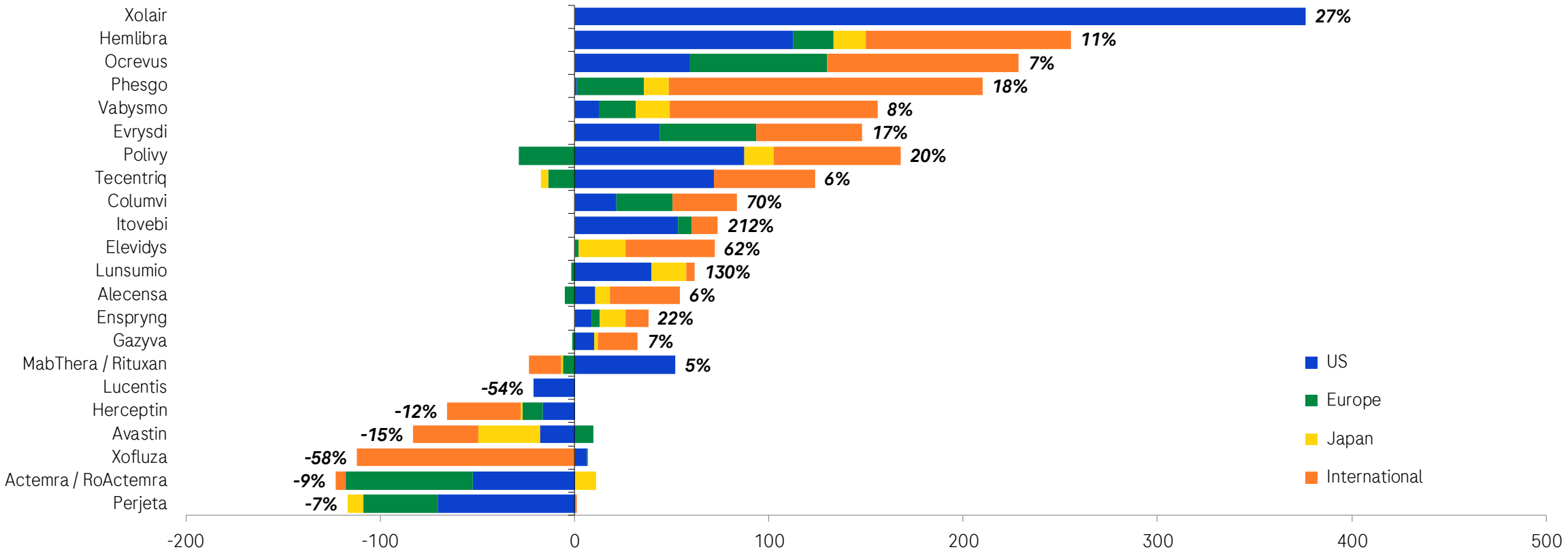
Includes shift of CHF ~105m (CER) to Corporate



CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

HY 2026: Diversified set of key growth drivers

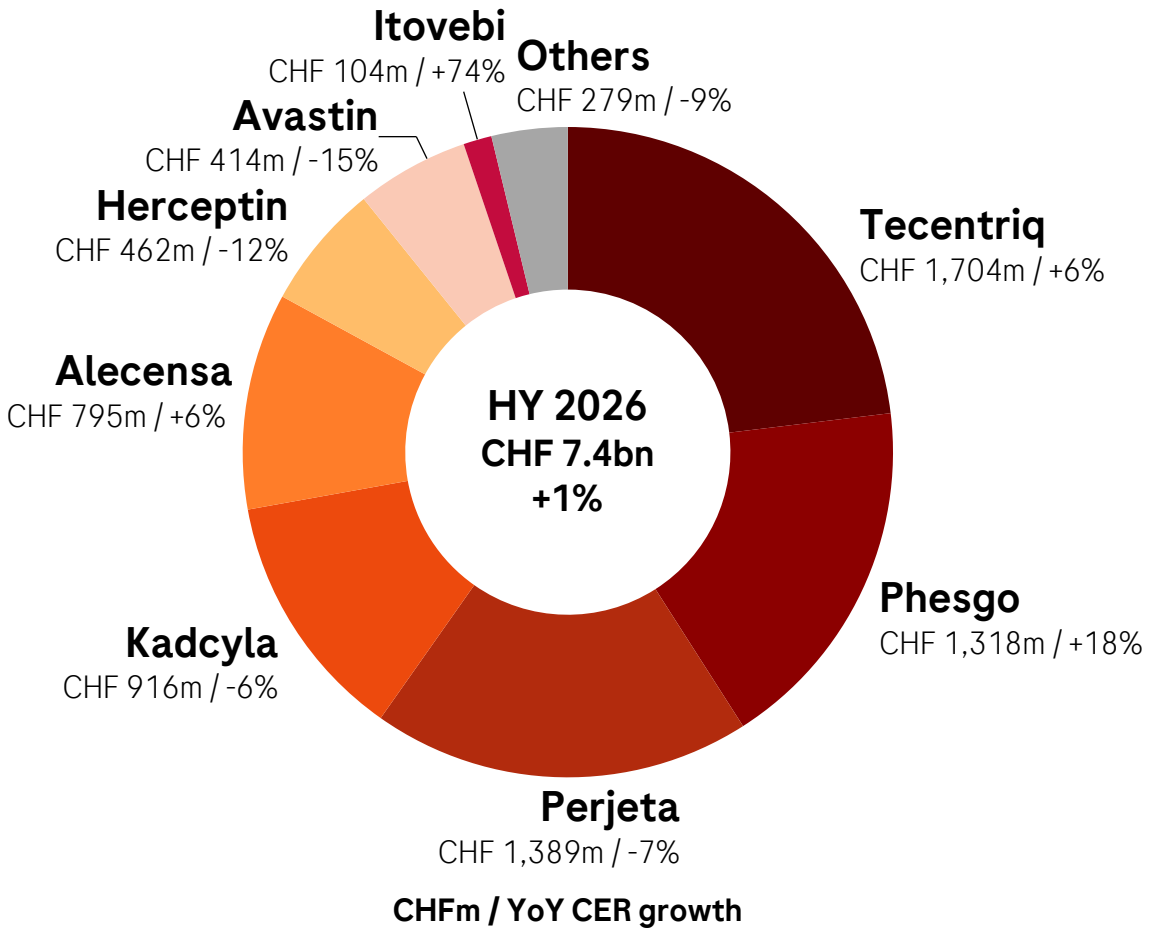
Xolair remaining top growth driver, closely followed by Ocrevus and Hematology franchise



Absolute values and growth rates at CER: Constant exchange rates (avg. full year 2025)

Oncology growth driven by Phesgo and Tecentriq

Giredestrant: lidERA PDUFA set for 30th Nov; positive Ph III for divarasib in 2L+ KRAS G12C+ NSCLC



Q2 update

- Phesgo: Global conversion rate at 54%*
- Perjeta: Conversion to Phesgo ongoing
- Kadcyla: Competitive pressure in US & EU as expected
- Itovebi: Launch in 1L *PIK3CA*-mut HR+ BC ongoing
- Tecentriq: Growth driven by new indications; US approval in MIBC (IMvigor011) achieved; US & EU filing in dMMR CC (ATOMIC) achieved, US PDUFA set for 9th Oct
- Alecensa: Flat growth expected for FY
- Giredestrant in BC: PDUFA in adj. ER+/HER2- (lidERA) set for 30th Nov
- Divarasib: Positive Ph III (KRASCENDO 1) in 2L+ KRAS G12C+ NSCLC

Outlook 2026

- Giredestrant in adj. ER+/HER2- BC: US approval (PDUFA 30th Nov)
- Giredestrant in post CDKi ER+/HER2- mBC: US approval (PDUFA 18th Dec)
- Ph III (INAVO121) Itovebi in post CDKi *PIK3CA*-mut HR+/HER2- mBC
- Ph III (INAVO122) Itovebi + Phesgo in 1L *PIK3CA*-mut HER2+ mBC

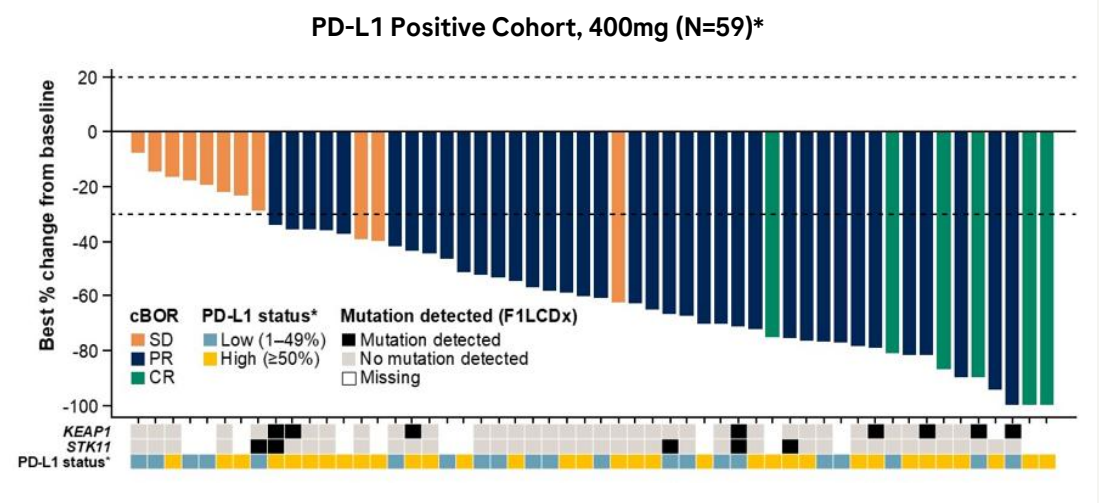
*Perjeta/Phesgo conversion rate calculated using volumes, currently taking 85 launch countries into account (83 countries at Q1 2026); CC: Colon cancer; CDKi: Cyclin dependent kinase inhibitor; CER: Constant exchange rates (avg. full year 2025); dMMR: Deficient mismatch repair; ER: Estrogen receptor; HER2: Human epidermal growth factor 2; HR: Hormone receptor; (m)BC: (Metastatic) breast cancer; NSCLC: Non-small cell lung cancer; MIBC: Muscle-invasive and metastatic bladder cancer; PDUFA: Prescription drug user fee act; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated

Divarasib in 2L+ NSCLC: Superiority in PFS and OS vs. SoC

Potentially best-in-class KRAS G12C inhibitor; strong divarasib activity in 1L NSCLC in combination with pembro

Ph II KRASCENDO 170 efficacy[1]

2026 ASCO ANNUAL MEETING



- PD-L1 positive cohort[2], cORR of 73% (CR: 10%), and mPFS of 19.3m
- PD-L1 negative cohort, unconfirmed ORR of 70% (PFS immature with median follow-up of 3.4m)
- Manageable safety profile; most common TRAEs were low grade, manageable and reversible
- Ph III (KRASCENDO 2) in 1L NSCLC: divarasib + pembro vs chemo + pembro ongoing

Roche RAS-targeted pipeline

Molecule	Indication	Ph I	Ph II	Ph III	Status
divarasib	2L+ mNSCLC H2H vs sotorasib/adagrasib	KRASCENDO 1			✓
divarasib + pembro	1L mNSCLC		KRASCENDO 2		
divarasib	eNSCLC (adjuvant)			KRASCENDO 3	FPI exp. H2 2026
KRAS G12Di	Solid tumors				
panRASi	Solid tumors				
pan-KRASi	Solid tumors				

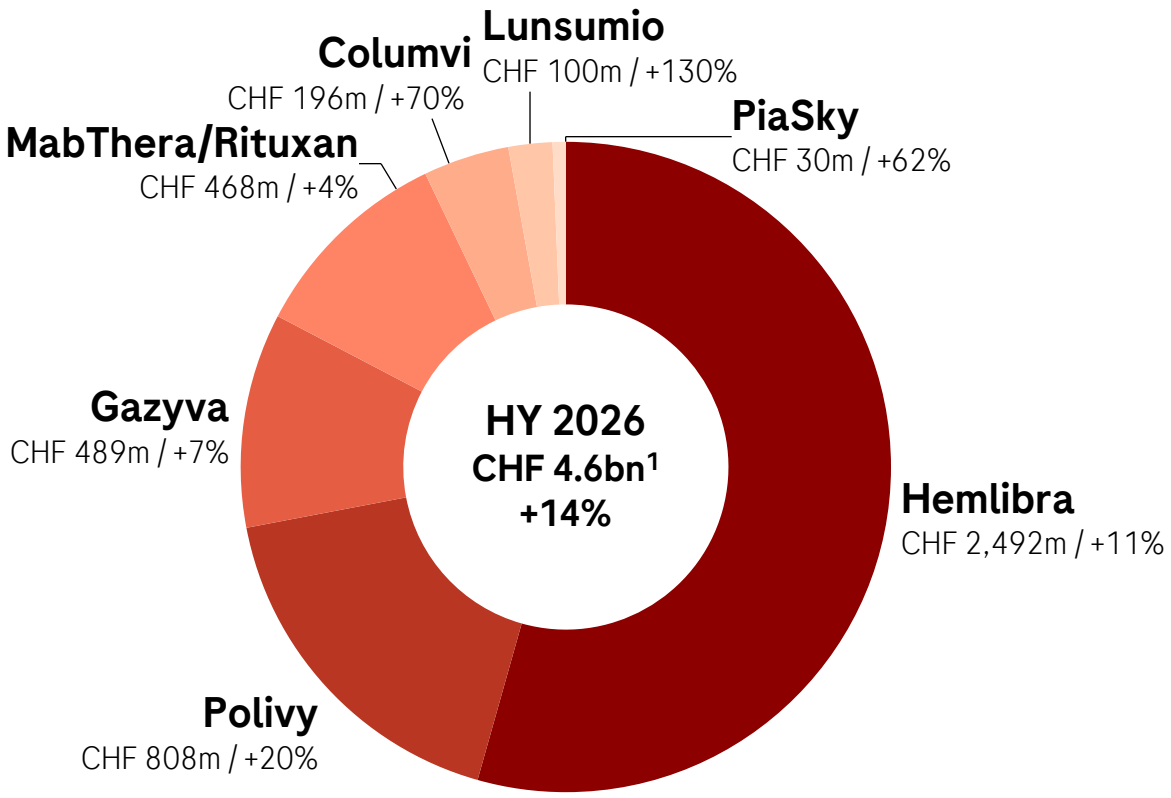
✓ Primary endpoint met

- Divarasib in 2L+ NSCLC: Superiority in PFS and OS vs. approved KRAS G12Ci demonstrated in Ph III H2H; OS significance reached at interim
- Broad Ph III program for divarasib in NSCLC covering adjuvant, 1L (chemo-free pembro combo), and 2L+ (H2H vs sotorasib/adagrasib)
- RG6620 (KRAS G12Di) potential for differentiation based on potency and selectivity vs other G12Di

1. Skoulidis et al., ASCO 2026; 2. All pts with a post-baseline assessment with tumor size reduction; *3 patients did not receive treatment, 1 patient received divarasib 200 mg; (c)ORR: (Confirmed) objective response rate; CR: Complete response; H2H: Head-to-head; (e)NSCLC: (Early) non-small cell lung cancer; (m)PFS: (Median) progression free survival; (TR)AE: (Treatment-related) adverse event

Polivy expanding strong global position in 1L DLBCL (IPI 0-5)

Lunsumio in 2L+ DLBCL (SUNMO) with US PDUFA set for 9th Feb



CHFm / YoY CER growth

Q2 update

- Hemlibra: Continued global growth; increasing adoption in non-inhibitor patients
- Polivy: Strong 1L DLBCL uptake with >120k pts treated globally
- Gazyva: Growth despite increasing competitive pressure in CLL and FL
- Columvi: Driven by 2L+ DLBCL
- Lunsumio: Strong 3L+ FL uptake; US filing in 2L+ DLBCL (SUNMO) achieved with PDUFA set for 9th Feb
- PiaSky in aHUS: Positive Ph III (COMMUTE) presented at ERA

Outlook 2026

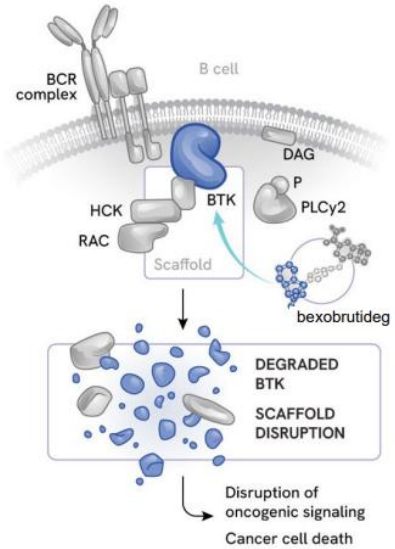
- PiaSky in aHUS: US/EU filing
- Ph III (CELESTIMO) Lunsumio + lenalidomide in 2L+ FL

1. Venclexta sales booked by AbbVie and therefore not included; aHUS: Atypical hemolytic uremic syndrome; CER: Constant exchange rates (avg. full year 2025); CLL: Chronic lymphocytic leukemia; DLBCL: Diffuse large B cell lymphoma; FL: Follicular lymphoma; PDUFA: Prescription drug user fee act

Bexobrutideg: Potential BIC BTK-targeted therapy

Broad development program in Hematology; ongoing expansion to Neurology and Immunology indications

Bexobrutideg (BTK degrader)



- Validated target for hematologic malignancies, as well as in Neurology and Immunology
- Oral, CNS-penetrant, highly potent and selective BTK degrader[1]
- Degradation removes all functions of BTK, unlike BTK inhibition, and overcomes BTKi resistance mutations[2-4]

Development program

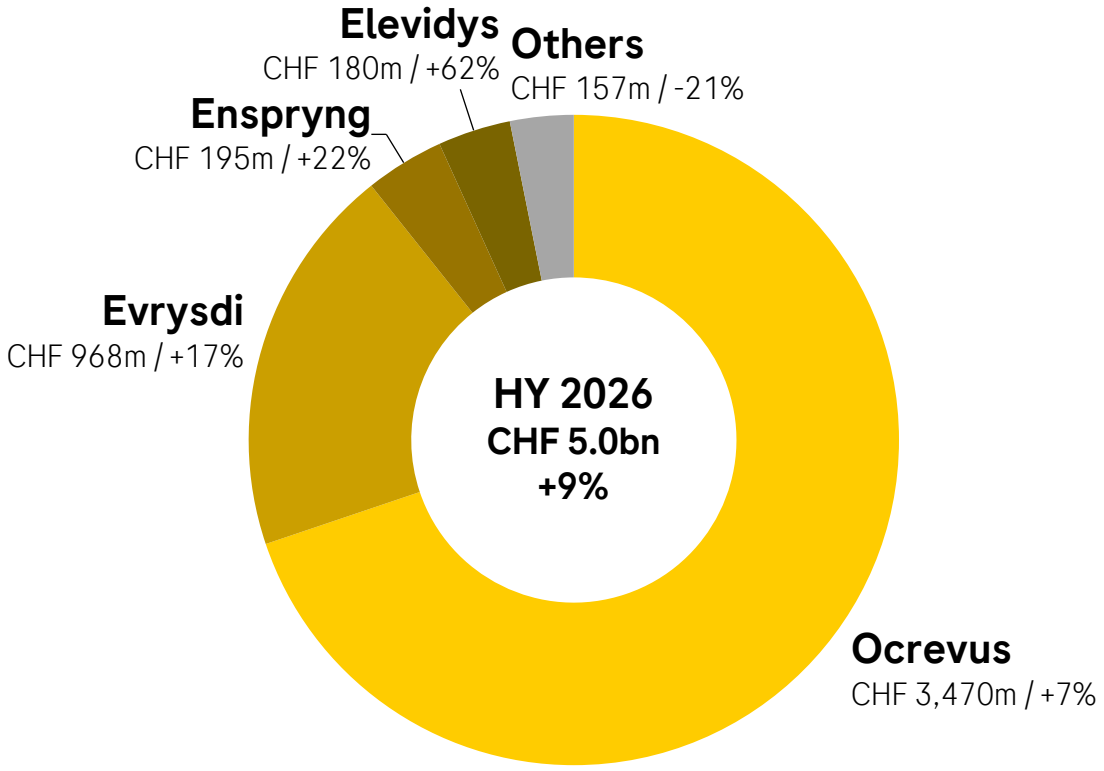
TA	Indication	Ph I	Ph II	Ph III	Status
Hematology	3L+ CLL	DAYBreak CLL-201			Ongoing
	2L+ CLL	DAYBreak CLL-306			Ongoing
	1L+ CLL / NHL	Basket combination trial*			Ongoing
	MCL				Ph III planned
	WM				Ph III planned
Neurology	MS				Ph II planned
Immunology	CSU				Ph II planned

- Broad development program in malignant hematology, incl. multiple lines of therapy and combinations in B-Cell malignancies; Ph III in 2L+ CLL and Ph I/II in 1L+ CLL have been initiated
- Planning to initiate Ph II trial in MS and CSU based on strong scientific rationale and preclinical findings for bexobrutideg[5,6]
- Evaluating initiation of additional Ph III trials, including combinations in CLL

1. Noviski. 8th TPD & Induced Proximity Summit, 2025; 2. Noviski et al. 20th Biennial International Workshop on CLL, 2023; 3. Hansen. TPD Basel, 2023; 4. Montoya et al. Science 2024; 383; eadi5798; 5. Noviski et al. ACR Convergence, 2024; 6. Noviski et al. SID Annual Meeting, 2026; *Ph Ib/II trial; BTK: Bruton's tyrosine kinase; CLL: Chronic lymphocytic leukemia; CNS: Central nervous system; CSU: Chronic spontaneous urticaria; NHL: Non-Hodgkin's Lymphoma; MCL: Mantle cell lymphoma; MS: Multiple sclerosis; WM: Waldenström macroglobulinemia; In partnership with Nurix Therapeutics

Ocrevus: Global growth driven by SC

Ocrevus Zunovo is the fastest growing aCD20 MS treatment in the US



CHFm / YoY CER growth

Q2 update

- Ocrevus: ~44,000 patients on Ocrevus SC globally with significant momentum vs. Q1
  - US: ~60% of SC volume coming from community practices
- Evrysdi: Expanding strong position in SMA
- Trontinemab in AD: Updates shared at AAIC
  - New long-term safety, amyloid removal and biomarker data from Ph Ib/IIa (Brainshuttle™ AD) OLE
  - Ph III (PrevenTRON) study design in preclinical AD

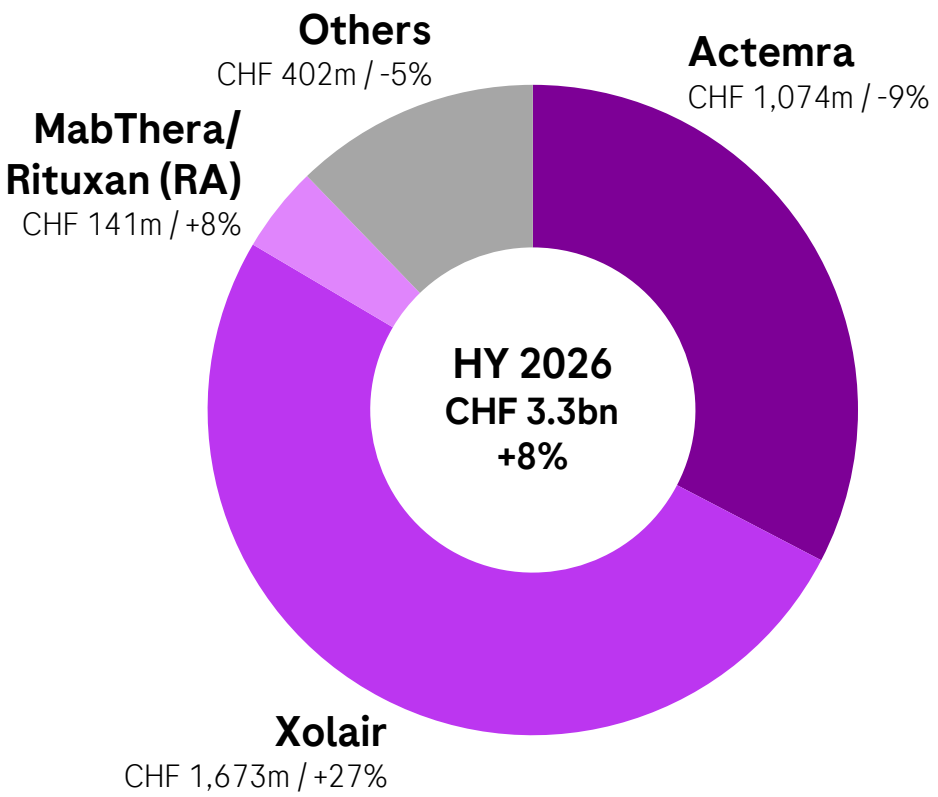
Outlook 2026

- Fenebrutinib in RMS/PPMS: US/EU filing
- Enspryng in MOGAD: US/EU filing

CER: Constant exchange rates (avg. full year 2025); AD: Alzheimer's disease; MOGAD: Myelin oligodendrocyte glycoprotein antibody-associated disease; (R/PP)MS: (Relapsing/Primary progressive) multiple sclerosis; SC: Subcutaneous; OLE: Open-label extension; SMA: Spinal muscular atrophy

Xolair driving overall Immunology growth

Gazyva: US & EU filing for MN achieved with PDUFA set for 15th Nov; US PDUFA in INS set for 27th Sep



CHFm / YoY CER growth

Q2 update

- Xolair: Growth driven by food allergy
  - Food allergy: >130,000 patients treated since launch
  - Biosimilar launch expected H2 2026
- Actemra: Biosimilar impact in US, EU and International
- Gazyva:
  - INS: US PDUFA set for 27th Sep & EU filing achieved
  - MN: US filing achieved/PDUFA set for 15th Nov & EU filing achieved; FDA BTD granted; Ph III (MAJESTY) data presented at ERA

Outlook 2026

- Gazyva in SLE: US approval (PDUFA 4th Dec)
- Gazyva in INS: US approval (PDUFA 27th Sep)
- Gazyva in MN: US approval (PDUFA 15th Nov)
- Ph III (IMAgINATION) sefaxersen in IgAN interim analysis

CER: Constant exchange rates (avg. full year 2025); BTD: Breakthrough therapy designation; IgAN: IgA nephropathy; INS: Childhood onset idiopathic nephrotic syndrome (also known as pediatric nephrotic syndrome (PNS); MN: Membranous nephropathy; PDUFA: Prescription drug user fee act; SLE: Systemic lupus erythematosus

Gazyva: Potential to become first approved treatment for MN

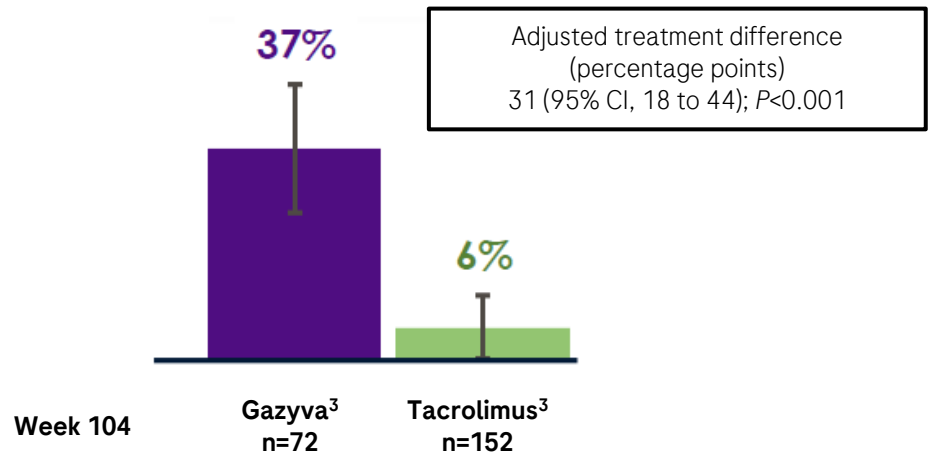
Gazyva demonstrated superiority over tacrolimus in CR at week 104; all Gazyva indications filed in US & EU

Ph III (MAJESTY) in MN results[1,2]

63rd ERA CONGRESS

The NEW ENGLAND JOURNAL of MEDICINE

Primary endpoint: CR[3] (UPCR ≤0.3 g/g and stable eGFR (≥85% of baseline))



- Primary endpoint met: 37% patients on Gazyva achieved complete remission[3] vs. 6% on tacrolimus at week 104
- Superior to tacrolimus on key secondary endpoints, incl. overall remission[4] (51% vs. 13%) at week 104
- Safety consistent with well-characterized Gazyva profile

Development program

Indication	Ph I	Ph II	Ph III	Status
LN			REGENCY	✓ Approved in US/EU
SLE			ALLEGORY	✓ Filed in US/EU
MN			MAJESTY	✓ Filed in US/EU
INS			INShore	✓ Filed in US/EU

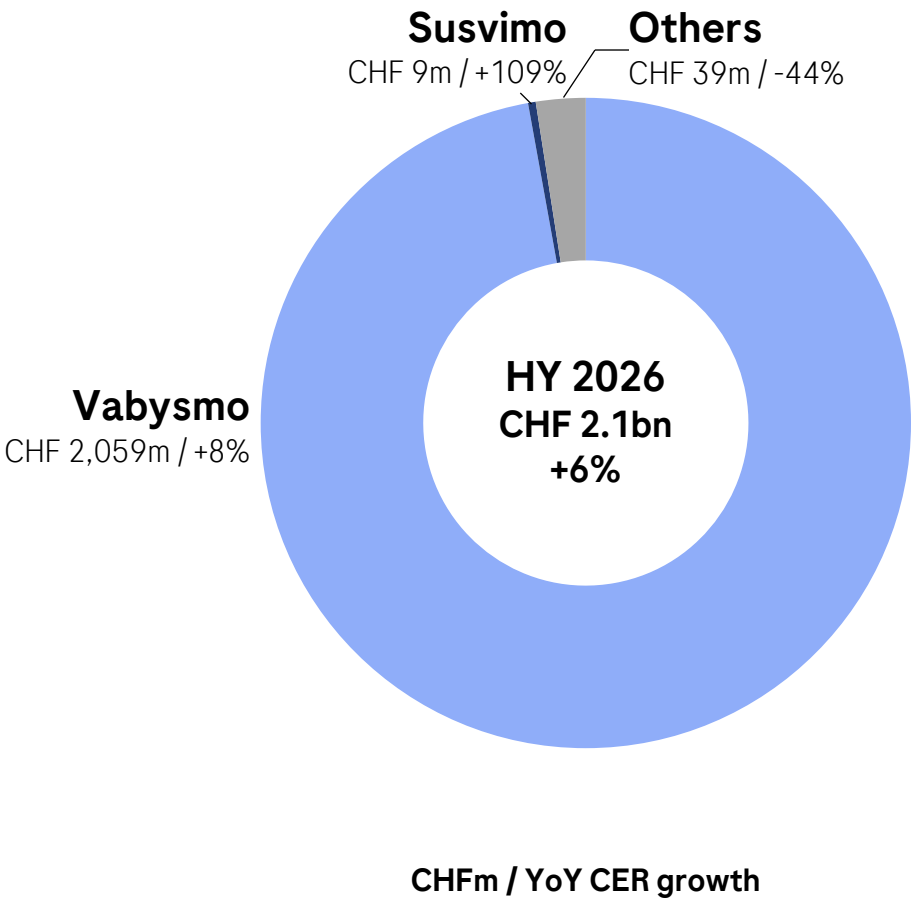
✓ Primary endpoint met

- LN launch in line with expectations
- US & EU filing in SLE achieved; PDUFA set for 4th Dec
- Positive MN results filed in US & EU; PDUFA set for 15th Nov; presented at ERA
- Positive INS results filed in US & EU; PDUFA set for 27th Sep
- FDA BTD and priority review granted for MN and INS

1. Fervenza FC et al., ERA Congress 2026; 2. Fervenza FC, et al. N Engl J Med. 2019; 381:36-46; 3. CR is defined as achievement of UPCR ≤0.3 g/g and eGFR ≤15% below baseline. eGFR is calculated using the Chronic Kidney Disease Epidemiology Collaboration equation. Patients experiencing intercurrent events of escape therapy, treatment failure or early study discontinuation were considered to be non-responders under the composite strategy; 4. Overall remission was defined as CR or PR at Week 104, wherein CR was defined as a UPCR of ≤0.3 g/g from a 24-hour urine collection and a stable eGFR defined as being no worse than 15% below the baseline value, calculated using the Chronic Kidney Disease Epidemiology Collaboration equation, and PR was defined as a ≥50% reduction in UPCR from baseline and ≤3.5 g/g but >0.3 g/g, with stable eGFR at Week 104. Data (UPCR or eGFR) were missing for 1 patient in the obinutuzumab group at Week 104 and were imputed using multiple imputation; BTD: Breakthrough therapy designation; CI: Confidence interval; CR: Complete remission; eGFR: Estimated glomerular filtration rate; INS: Childhood onset idiopathic nephrotic syndrome (also known as pediatric nephrotic syndrome (PNS); LN: Lupus nephritis; MN: Membranous nephropathy; PDUFA: Prescription drug user fee act; SLE: Systemic lupus erythematosus; UPCR: Urine protein-to-creatinine ratio

Vabysmo with continued global market share expansion

Enspryng in TED US filing and priority review achieved with PDUFA set for 15th Oct



Q2 update

- Vabysmo: Continued global growth and market share gains
  - US: >60% of patient starts are naïve
  - Positive Ph III (POYANG) in mCNV
- Enspryng in TED: US filing achieved; priority review granted

Outlook 2026

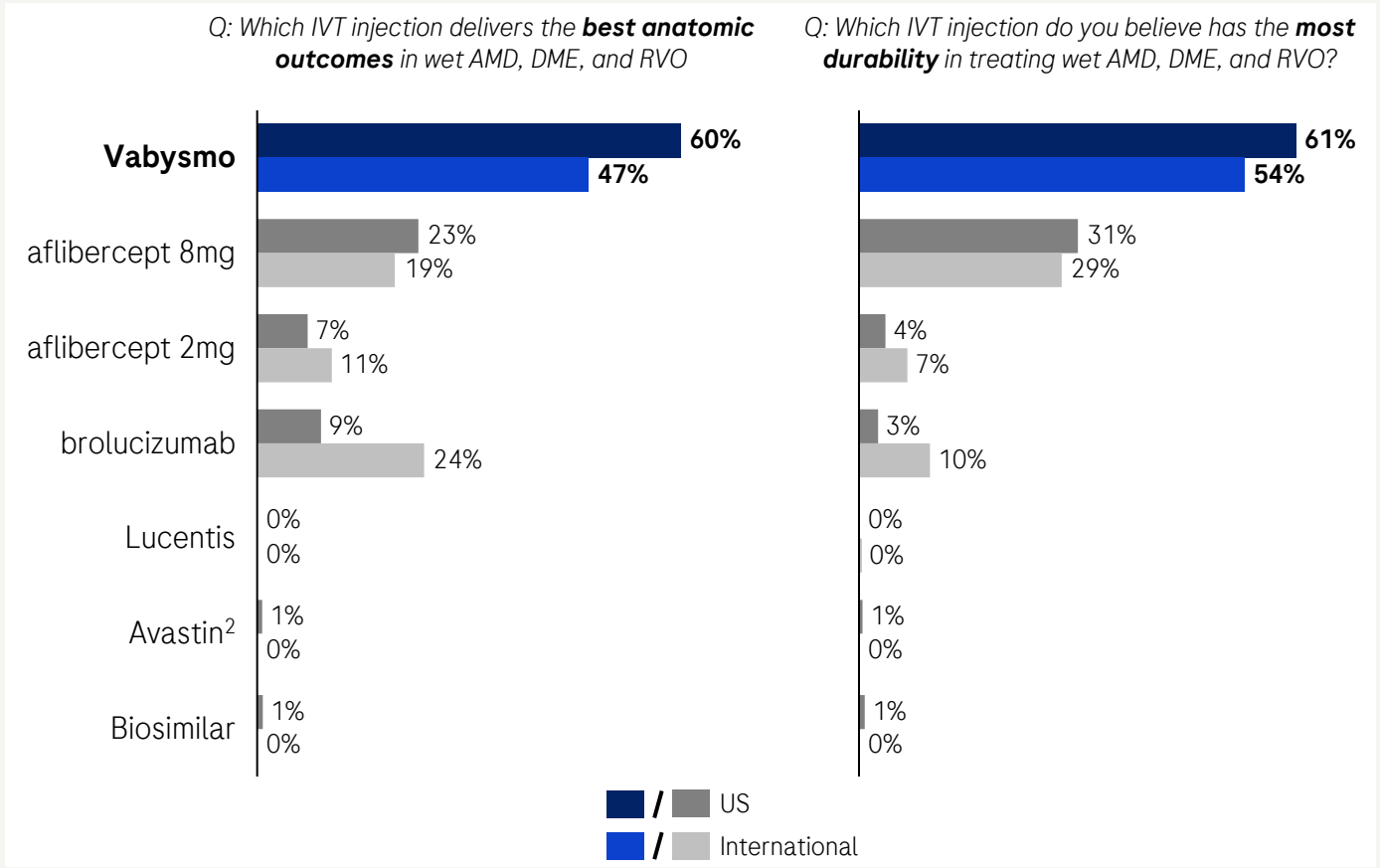
- Vamikibart in UME: US/EU filing
- Enspryng in TED: US approval (PDUFA 15th Oct)
- Susvimo in nAMD: EU approval

CER: Constant exchange rates (avg. full year 2025); mCNV: Myopic choroidal neovascularization; nAMD: Neovascular age-related macular degeneration; PDUFA: Prescription drug user fee act; RVO: Retinal vein occlusion; TED: Thyroid eye disease; UME: Uveitic macular edema

Vabysmo: Expecting continued global market share gains

Strong, consistent clinical and real-world data on anatomic outcomes and durability across indications

ASRS PAT survey: Anatomic outcomes and durability in AMD, DME & RVO[1]



Global growth outlook

US

- Branded market has stabilized
- Continued branded market share gains expected

EU

- Further share expansion across EU5 markets expected
- Strong volume growth to overcompensate pricing effects

ROW

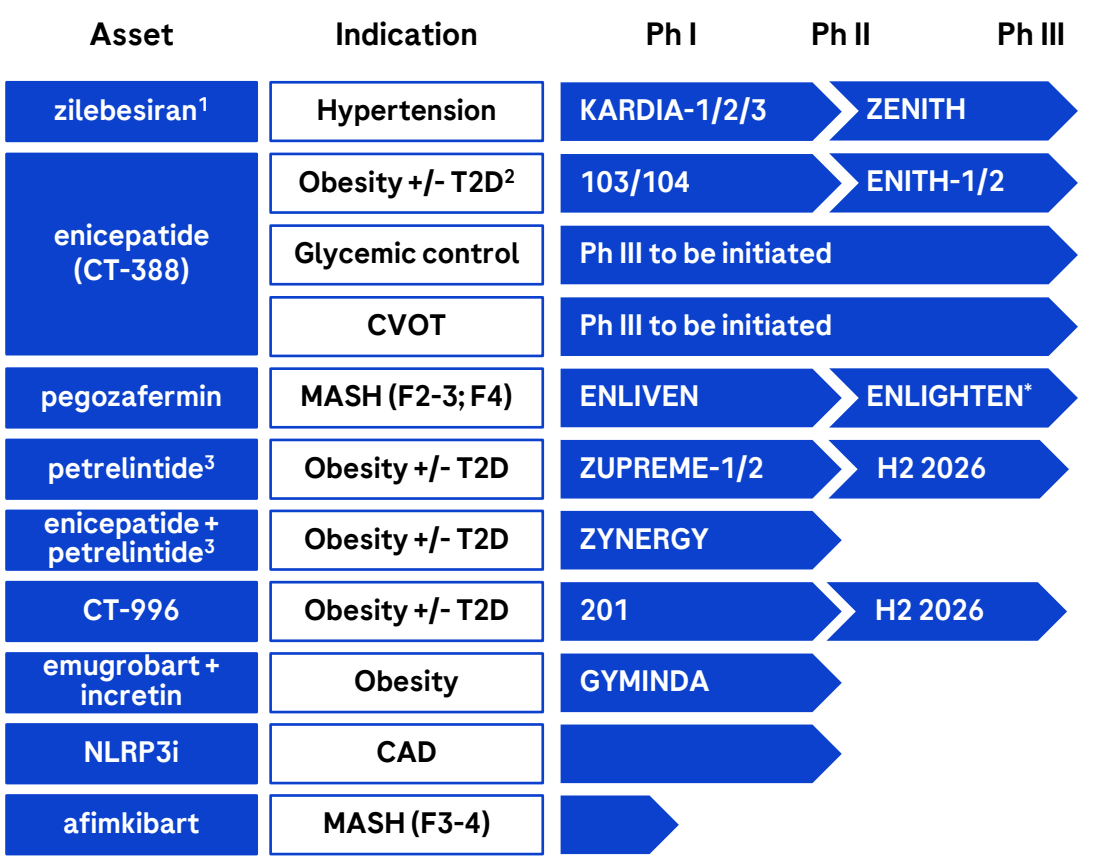
- Strong growth across key markets driven by market share expansion
- Significant growth potential remaining

1. Hahn P, ASRS PAT (Preferences and Trends) Membership Survey 2026, n=978 (anatomic outcomes), n=980 (durability), adapted visualization without changes to underlying data; 2. Avastin is not approved for use in the eye; ASRS: American society of retinal specialists; DME: Diabetic macular edema; nAMD: Neovascular age-related macular degeneration; Totals and subtotals may include differences due to rounding; Note: Survey data represents self-reported, subjective physician feedback and practice preferences. These observational findings do not constitute controlled clinical evidence and cannot be used to establish comparative clinical safety or efficacy.

CVRM pipeline progressing

Ph II data for enicepatide and petrelintide shared at ADA; pipeline progressing towards late-stage

Clinical development



Q2 update

- Enicepatide: Ph II (103) data in obesity presented at ADA
- Petrelintide: Ph II (ZUPREME-1) data in obesity presented at ADA
- Acmopatide (CT-868): Development will be discontinued as other T1D development program get prioritized

Outlook 2026

Results:

- Ph II (104) enicepatide in obesity with T2D
- Ph II CT-996 in obesity
- Ph II (ZUPREME-2) petrelintide in obesity with T2D
- Ph II (GYMINDA) emugrobart + tirzepatide in obesity

Pipeline progress:

- Enicepatide + petrelintide: Ph II (ZYNERGY) in obesity to be initiated
- Petrelintide: Ph III in obesity to be initiated
- CT-996: Ph III in obesity initiation planned

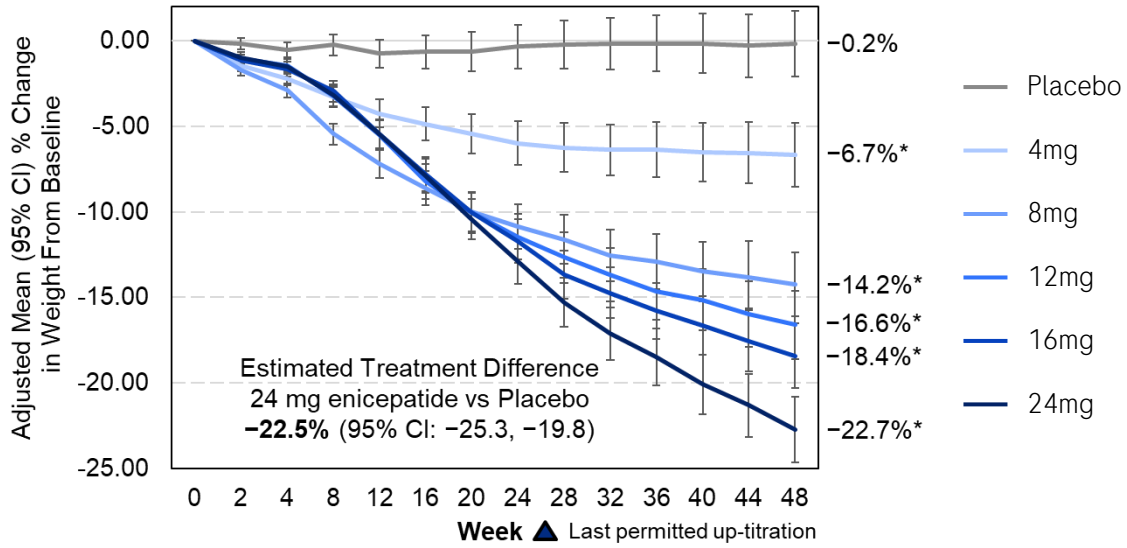
1. Zilebesiran in partnership with Alnylam; 2. Patients with obesity or overweight with at least one weight-related comorbidity including type 2 diabetes; 3. Petrelintide in partnership with Zealand Pharma; *Ph III program consisting of ENLIGHTEN fibrosis in MASH (F2-F3) and ENLIGHTEN cirrhosis in MASH (F4); CAD: Coronary arterial disease; CVOT: Cardiovascular outcomes trial; MASH: Metabolic dysfunction-associated steatohepatitis; T2D: Type-2 diabetes

Enicepatide: Potential for BIC weight loss

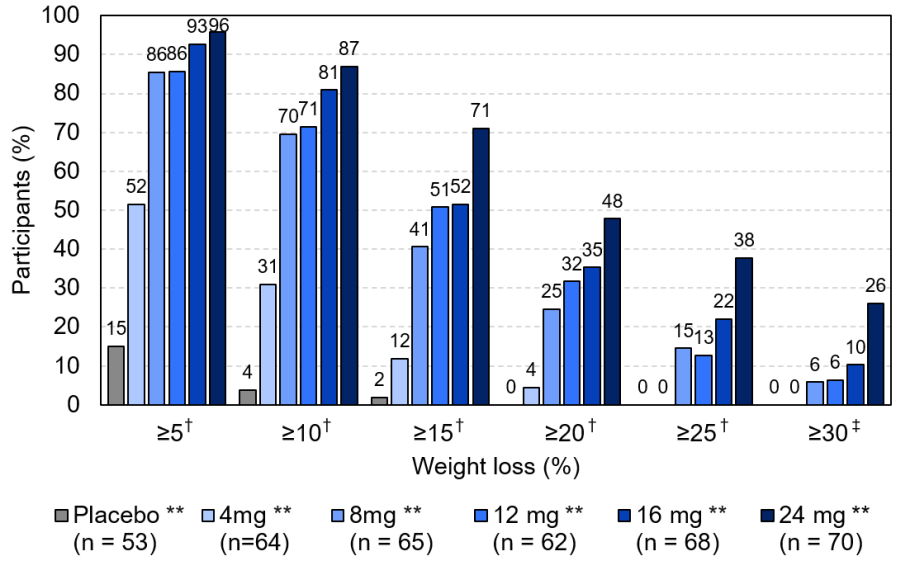
Broad Ph III development program in obesity, T2D and CVOT with additional trials under evaluation

Ph II (103) results in overweight/obesity[1]

Weight loss (efficacy estimand)



Weight reduction thresholds (at 48 weeks)



- No weight loss plateau reached at 48 weeks for 24mg dose, indicating potential to achieve additional weight loss with longer treatment; at 24mg 48% of patients achieved >20% weight loss, 38% > 25% weight loss and 26% > 30% weight loss
- Tolerability profile generally consistent with other incretin-based therapies at similar development stage
- Ph III (ENITH-1/2) in obesity ongoing; Ph III trials in glycemic control and CVOT to be initiated; Ph II (ZYNERGY) for enicepatide + petrelintide in obesity to be initiated

1. Lingvay I et al., ADA 2026, Poster-2813-LBA; *p ≤ 0.001 vs placebo; **The denominator to calculate the proportion is based on the number of participants who have reached week 48.
†Prespecified endpoint; BIC: Best in class; CI: Confidence interval; CVOT: Cardiovascular outcomes trial; T2D: Type 2 diabetes

2026: Key newsflow outlook*

Giredestrant + everolimus (post CDKi ER+/HER2- mBC) filed in US; 5 FDA priority reviews achieved

	Compound	Indication	Milestone		
Regulatory	**giredestrant**	adjuvant ER+/HER2- early BC	US/EU filing	✓	*(US)*
	giredestrant + everolimus	post CDKi ER+/HER2- mBC	EU filing/US approval		
	Gazyva	SLE	US/EU filing	✓	*(US&EU)*
	Gazyva	INS	US/EU filing	✓	*(US&EU)*
	Enspryng	MOGAD	US/EU filing		
	Enspryng	TED	US filing	✓	
	vamikibart	UME	US/EU filing		
	Susvimo	nAMD	EU approval		
	Susvimo	DME	EU filing		*2027*
	PiaSky	aHUS	US/EU filing		
	Lunsumio + Polivy	2L+ DLBCL	US approval		*2027*
Clinical results	**divarasib**	2L+ KRAS G12C+ NSCLC	Ph III KRASCENDO 1	✓	
	giredestrant + palbociclib	1L ER+/HER2- mBC	Ph III persevERA	✗	
	Itovebi + fulvestrant	post CDKi *PIK3CA*-mut HR+/HER2- mBC	Ph III INAVO121		
	Itovebi + Phesgo	1L *PIK3CA*-mut HER2+ mBC	Ph III INAVO122		
	Lunsumio + lenalidomide	2L+ FL	Ph III CELESTIMO		
	Enspryng	MOGAD	Ph III METEOROID	✓	
	fenebrutinib	RMS	Ph III FENhance 1/2	✓	
	Gazyva	MN	Ph III MAJESTY	✓	
	sefaxersen	IgAN	Ph III IMAgINATION		
	Vabysmo	mCNV	Ph III POYANG	✓	
	Evrysdi + emugrobart	SMA	Ph II MANATEE	✗	
	emugrobart	FSHD	Ph II MANOEUVRE	✗	
	emugrobart + tirzepatide	Obesity	Ph II GYMINDA		
	enicepatide (CT-388) (QW SC)	Obesity	Ph II	✓	
	acmopatide (CT-868) (QD SC)	T1D with Obesity	Ph II	✓	
	CT-996 (QD oral)	Obesity	Ph II		
	petrelintide	Obesity	Ph II	✓	

Additional 2026 newsflow: ✓ **Tecentriq** US approval in MIBC

*Outcome studies are event-driven: timelines may change



Diagnostics Division

Matt Sause

CEO Roche Diagnostics

HY 2026: Diagnostics sales

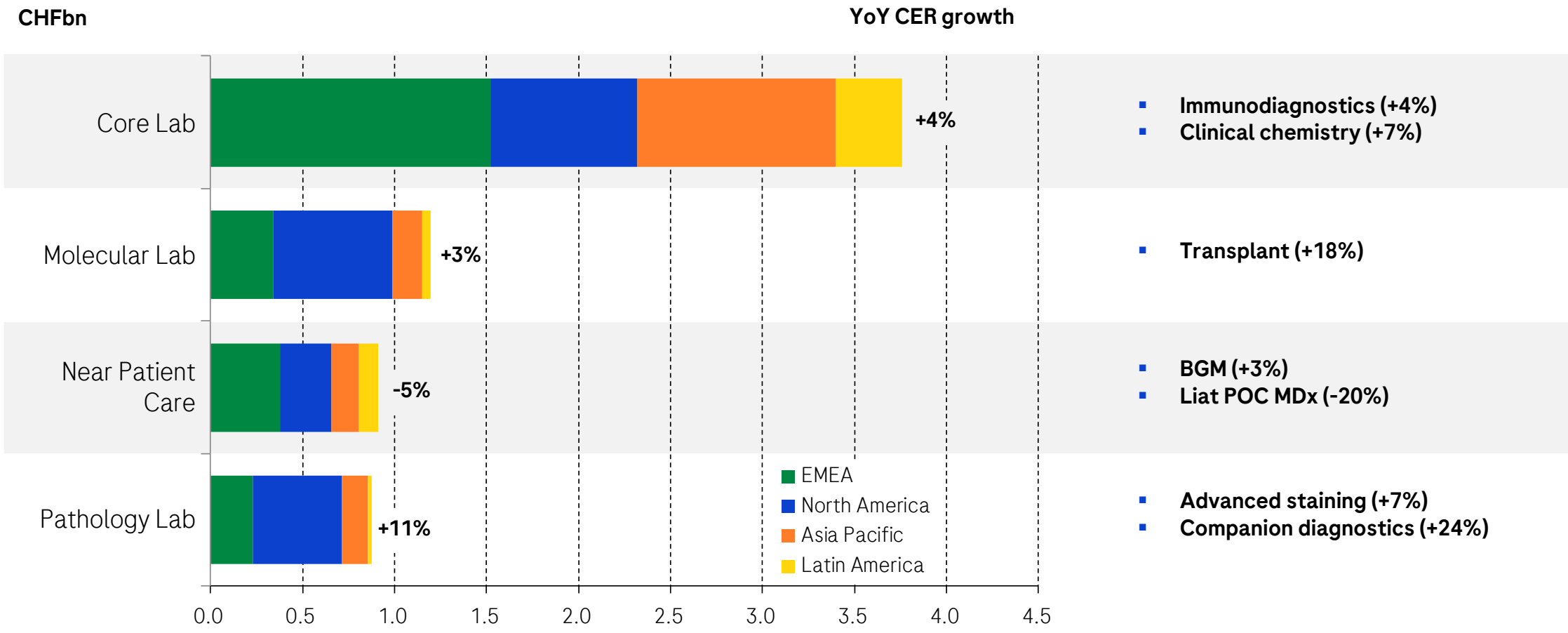
Diagnostics growing at +3%, impacted by China healthcare pricing reforms

	HY 2026 CHFm	HY 2025 CHFm	Change in % CHF	Change in % CER
Diagnostics Division	**6,735**	**6,959**	**-3**	**3**
Core Lab	3,756	3,839	-2	4
Molecular Lab	1,196	1,250	-4	3
Near Patient Care	909	1,018	-11	-5
Pathology Lab	874	852	3	11

CER: Constant exchange rates (avg. full year 2025)

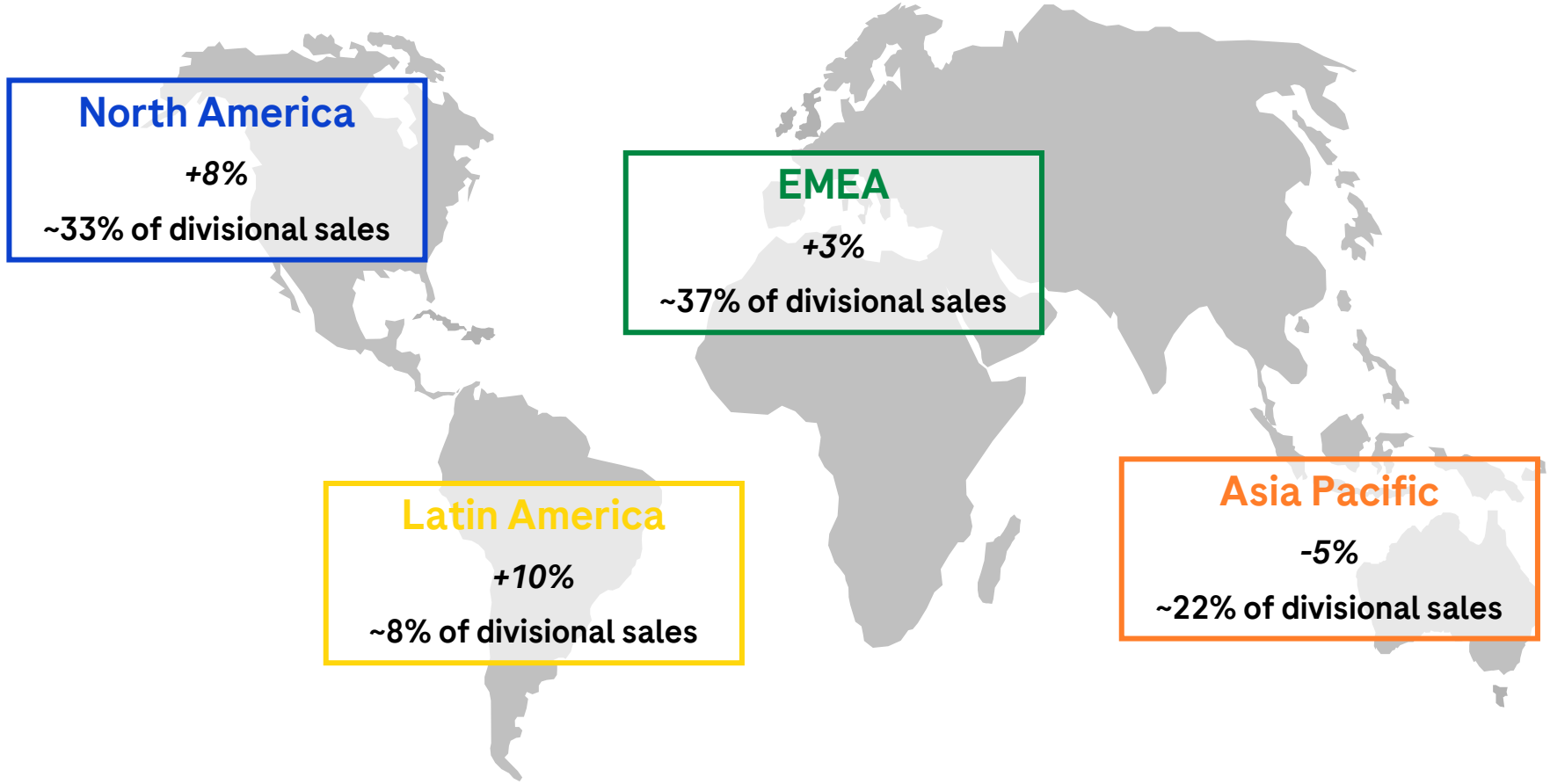
HY 2026: Diagnostics highlights

Diagnostics growing at +3%, impacted by China healthcare pricing reforms



BGM: Blood glucose monitoring; CER: Constant exchange rates (avg. full year 2025); EMEA: Europe, Middle East and Africa; POC MDx: Point of care molecular diagnostics

HY 2026: Diagnostics regional sales



Growth rates at constant exchange rates (avg. full year 2025); EMEA: Europe, Middle East and Africa

HY 2026: Diagnostics core operating profit

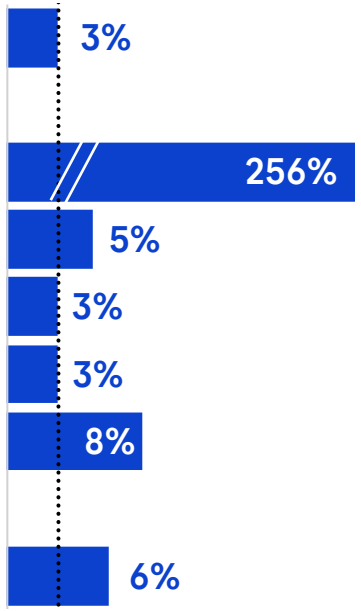
Core operating profit higher by +6%, driven by higher sales and other revenue

	2026	
	CHFm	Abs. CERm
Sales	**6,735**	**+228**
Other revenue	123	+90
Cost of sales	-3,442	-173
R&D	-870	-27
SG&A	-1,388	-43
OOI&E	22	+2
Core operating profit	**1,180**	**+77**
Core OP as % of sales	*17.5%*	
At CER		*18.0%*
		(HY 2025: 17.4%)

Includes shift of CHF ~24m (CER) to Corporate

2026 vs. 2025

CER growth



-6% in CHF

CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

AXELIOS 1 Sequencing platform launch

AXELIOS 1 delivers world-record speed, high accuracy, and multi-modal flexibility

Performance[1]

SBX-D	SBX-duplex (SBX-D) output (concordant duplex bases per 4 hr run)	**≥ 1.8 Tb**
	Mean insert length[2]	**~ 230 to 260 bp**
	Accuracy[3] (concordant duplex bases)	**~ Q38**
SBX-S	SBX-simplex output	**~40bn total reads (~175 bp insert length)**
	Insert length	**Up to 1500 bp[4]**
Maximum run time[5]	AXELIOS 1 Sequencing Instrument	**1 run: ~5.5 hr 4 runs (queued): ~22 hr**

Key partners

Hartwig Medical Foundation

wellcome sanger institute

Olink

BROAD INSTITUTE

10x GENOMICS

Market opportunity and differentiation

- The sequencing market was valued at USD 7.3bn in 2025[6]
- Demonstrated high performance in ultra-sensitive applications such as MRD detection, tumor/normal analysis, and WGS using SBX-duplex mode
- Efficient high-throughput single-cell and spatial omics for large-scale, read intensive research with SBX-simplex mode

AXELIOS 1 is for research use only; 1. Performance metrics, output, mean read length, quality score, etc. can vary based on sample type and quality, library prep conditions and loading concentration, and other experimental conditions 2. Includes post-consensus adapter-trimmed full length and partial duplex reads 3. Based on empirical accuracy calculated using BWA aligned bam files for high confidence regions of Genome in a Bottle (GIAB) samples; 4. SBX technology is capable of generating longer reads of up to ~1500bp (with a distribution from ~200bp to ~1500 bp) under appropriate sample and library prep conditions 5. Max run time for sequencing instrument includes pre-sequencing setup, sequencing time and post-run cleanup; 6. DeciBio. (2025). 2025 NGS Manufacturer Market Size, Growth and Trends (2022–2028); bp: Base pairs; MRD: Minimal residual disease; SBX-D: SBX-duplex; SBX-S; SBX-simplex; Tb: Terabases; WGS: Whole genome sequencing

PathAI acquistion[1]

Expanding our pathology portfolio with the best-in-class digital pathology solution

Roche Diagnostics oncology ecosystem[2]



PathAI overview

- PathAI is an AI-powered precision pathology company
- Its core offering includes a best-in-class Image Management System alongside a broad suite of diagnostic algorithms

Value to Roche Diagnostics

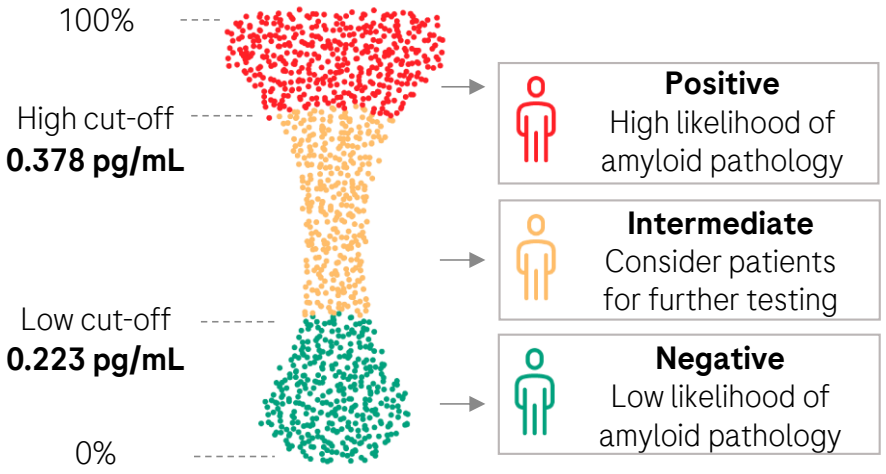
- PathAI Image Management Solution is complementary to Roche IHC and H&E, creating a highly differentiated solution
- AI-powered insights, including FDA approved algorithms, offer additional value to pathologists
- Roche's CDx leadership and PathAI's AI capabilities improve leading position with biopharma partners
- AI solutions enhance our Roche Pharma business by supporting clinical trials and introducing new biomarkers

Note: 1. Pending deal closure; 2. Roche ecosystem refers to internal assets, acquisitions, and partnerships; 3. AXELIOS is for research use only; CDx: Companion diagnostics; H&E: Hematoxylin and eosin; IHC: Immunochemistry; MRD: Minimal residual disease

Elecsys® pTau217

First blood-based biomarker to rule in and rule out Alzheimer's pathology in primary care

Elecsys® pTau217



PPV: 78.3% and NPV: 98.6% in primary care
PPV and NPV>90% in secondary care

Market needs and opportunity

- Alzheimer's is the leading cause of dementia, a condition projected to affect 80m people by 2030[1]
- Diagnosis takes three years after symptom onset and standard testing methods are invasive or not widely accessible[1]

Differentiation

- Enables rapid, minimally invasive and timely diagnosis via blood-based testing in primary care settings
- Validated in a real world cohort across settings and clinical stages demonstrating high diagnostics accuracy
- Fulfills CEOi consensus criteria for blood based biomarkers for rapid and safe triage and confirmatory diagnosis of patients

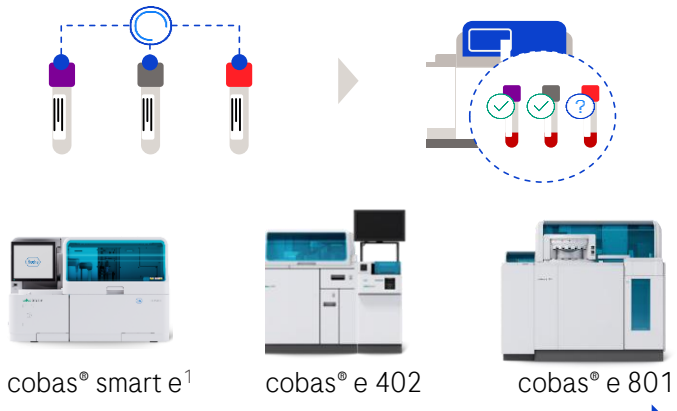
1. ADI's annual World Alzheimer's Report (2022); CEOi: Global CEO Initiative on Alzheimer's Disease; PPV: Positive predictive value; pTau: Phosphorylated tau; NPV: Negative predictive value

Elecsys® IGRA TB

Entering the attractive latent TB segment and strengthening Roche's Infectious Disease offering

46TH ANNUAL CONGRESS

Elecsys® IGRA TB



Performance		
Mixed risk cohort[3]	**Low risk cohort**[4]	**Active TB cohort**[5]
PPA: 91.12% NPA: 94.57%	Specificity: 95.32%	Relative sensitivity:[6] 100%

Market needs and opportunity

- ~25% of the global population may have been infected with the bacteria that cause TB[7]
- Expanding access to diagnostics for latent TB infection is essential to breaking the cycle of progression and transmission

Differentiation

- On par performance with standard of care and high intermediate precision and repeatability (CV < 2%)
- Improved workflow vs. standard of care, with faster turnaround and scalable throughput on the large Elecsys installed base
- Enables future automation of the pre-analytical steps with cobas ultra solutions[8]

1. In development; 2. Scalable to up to 4 e801 analytical units (400 IGRA samples/hour) 3. Intended use population: individuals seeking TB infection testing in low- and high-TB burden settings in the European and Western Pacific regions (n=1852); 4. Apparently healthy individuals without known risk factors for TB infection, living in a low-burden setting in the European region (n=449); 5. Not intended use population: confirmed active TB patients in low- and high-TB burden settings in European, Americas, and African region (n=78); WHO guidelines recommend against testing in this population, cohort was necessary to estimate sensitivity due to lack of a gold standard; 6. Relative to standard-of-care/established IGRA methods 7. WHO tuberculosis Fact Sheet November 2025; 8. Technical feasibility, availability beyond 2028; CV: Coefficient of variation; NPA: Positive predictive agreement; PPA: Positive predictive agreement; TB: Tuberculosis

Diagnostics key launches 2026

	Area	Product	Description	Market	Status
Systems	**Molecular Lab**	AXELIOS 1 Sequencing Solution	Novel high-throughput solution for ultra-rapid, scalable, and flexible sequencing powered by Sequencing by Expansion (SBX) technology	Global	✓
Tests	**Core Lab**	Elecsys® pTau 217	Non-invasive blood-based biomarker to enable earlier detection, rule in and rule out Alzheimer's amyloid pathology	CE	✓
		Elecsys® NfL	Non-invasive blood-based biomarker to enable monitoring of disease activity in multiple sclerosis	CE	✓
		Elecsys® ApoE4p	Non-invasive blood-based biomarker to identify ApoE4 carrier, the strongest genetic risk factor for the development of Alzheimer's disease	CE	✓
		Elecsys® IGRA TB	Interferon-Gamma Release Assay (IGRA) to detect latent tuberculosis (TB) infection	CE	✓
	Molecular Lab	cobas® MPX-E	Molecular PCR test for the detection of HIV, HCV, HBV, and HEV in one assay for Donor Screening	CE	✓
		cobas® HDV	Molecular PCR test for aid in diagnosis of initial Hepatitis D virus infection and therapy monitoring	CE	✓
		eplex gastrointestinal panel	Multiplex Molecular PCR assay for the comprehensive detection of 19 potential pathogens associated with gastrointestinal infections	CE	
	Near Patient Care	cobas® liat lesion panel EUA	Rapid test to enable accurate detection and differential diagnosis of patients presenting with cutaneous and mucocutaneous lesions/ulcers to enable timely treatment and management. Supporting mpox health emergency	US EUA	
	Pathology Lab	VENTANA® BAP1	Highly specific marker for differentiating mesothelioma from benign mesothelial proliferation	CE	
Digital solutions	**Healthcare Insights**	LungFlag algorithm	Machine learning algorithm that identifies patients at high risk for lung cancer	CE	

AXELIOS 1 is for research use only; ApoE4p: Apolipoprotein E4 protein; BAP1: BRCA1 Associated Protein-1; HCV: Hepatitis C Virus; HBV: Hepatitis B Virus; HDV: Hepatitis D Virus; HEV: Hepatitis E Virus; MPX-E: Multiplex HEV; NfL: Neurofilament light chain; PCR: Polymerase Chain Reaction; SBX: Sequencing by Expansion

IR events in 2026

Additional events driven by readouts

Neurology Update 22 Apr

- MS franchise update at AAN
- Focus on fenebrutinib (FENhance 1&2) data in RMS

Diagnostics Day 12 May

- Group and Diagnostics division strategy updates
- Launch updates for mass spectrometry, SBX sequencing, and CGM
- Foundation Medicine strategy and highlights
- Portfolio updates on Core Lab, Molecular Lab, Near Patient Care, and Pathology Lab

Oncology Update 2 Jun

- Oncology franchise update at ASCO
- Focus on giredestrant key data (lidERA, persevERA, evERA) and clinical development program update
- Additional details on the oncology pipeline

CVRM Update 8 Jun

- Obesity franchise update at ADA
- Focus on obesity key data (enicepatide Ph II, petrelintide Ph II) and clinical development program update

Pharma Day 28 Sep

- Update on Pharma strategy and business performance
- Update on R&D excellence
- Update on AI in R&D
- Deep-dive into the current product portfolio
- Building blocks for future growth: Late-stage portfolio update

Neurology Update ✓ Virtual Mon, 9 Feb 16:30-17:30 CET	**Immunology Update** ✓ Virtual Mon, 9 Mar 17:00-18:00 CET	**Neurology Update** ✓ Virtual Wed, 22 Apr 17:00-18:00 CEST	**Diagnostics Day** ✓ London & virtual Tue, 12 May 13:00-16:20 BST	**Oncology Update** ✓ Chicago & virtual Tue, 2 Jun 14:00-15:30 CDT	**CVRM Update** ✓ Virtual Mon, 8 Jun 17:00-18:30 CEST	**Pharma Day** London & virtual Mon, 28 Sep 09:00-15:00 BST

AD: Alzheimer's disease; CGM: Continuous glucose monitor; CVRM: Cardiovascular, renal and metabolism; RMS: Relapsing multiple sclerosis; SBX: Sequencing by expansion

Doing now what patients need next

Changes to the development pipeline

Q2 2026 update

New to phase I

4 NMEs:

RG6697 NME - DME
RG6852 NME - mCRPC
RG6733 NME - MM
RG6441 NME - DR

New to phase II

3 NMEs:

RG6596 HER2 TKI (ZN-1041) – 2L+ HER2+ mBC
RG6418 selnoflast (NLRP3i) – coronary artery disease
CHU anti-HLA-DQ2.5 x gluten peptides antibody - celiac disease

2 AIs:

RG7828 Lunsumio - SLE
RG6114 Itovebi + ribociclib + fulvestrant – 1L HR+ Chr8P-loss PIK3CA-wt mBC

New to phase III

3 NMEs:

RG6512 zemocimig (NXT007) – hemophilia A
RG6160 cevostamab – 2-4L r/r MM
RGxxxx[+] bexobrutideg – r/r CLL

New to registration

2 AIs:

RG7159 Gazyva - membranous nephropathy
RG7446 Tecentriq – stage III dMMR/MSI-H colon cancer

1 AI (US):

RG6168 Enspryng - TED

Removed from phase I

4 NMEs:

RG6344 mosperafenib (BRAFi) - solid tumors
RG6382 CD19 x CD3 - SLE
RG6496 HTT SNP ASO - Huntington's disease
RG6468 NME – solid tumors

1 AI:

RG6418 selnoflast (NLRP3i) - inflammation

Removed from phase II

3 NMEs:

RG6351 anti-Tie2 agonist - DME
RG6641 acmopatide – T1D
RG6042 tominersen – Huntington's disease

Removed from phase III

Approvals

1 AI (US):

RG7446 Tecentriq - ctDNA-pos high-risk MIBC

Status as of July 23, 2026

[+]RGxxxx number to be confirmed

Roche Group development pipeline

Phase I (37 NMEs + 6 AIs)

RG6026	Columvi monotherapy + combos	heme tumors
RG6171	giredestrant monotherapy + combos	solid tumors
RG6330	divarasib monotherapy + combos	solid tumors
RG6411	enzelkitug (anti-CCR8)	solid tumors
RG6505	PanRAS inhibitor	solid tumors
RG6537	AR degrader	mCRPC
RG6538	P-BCMA-ALLO1	r/r multiple myeloma
RG6540	P-CD19 x CD20 - ALLO1	heme tumors
RG6620	KRAS G12D inhibitor	solid tumors
RG6648[1]	cMET ADC	solid tumors
RG6733	-	MM
RG6735	-	heme tumors
RG6794	CDK4/2i	HR+ HER2- BC
RG6795	CDK4i	HR+ HER2- BC
RG6810[2]	DLL3 ADC	SCLC
RG6852	-	mCRPC
RG6919[1]	B7H3 ADC	solid tumors
RG6975[3]	CDH17 ADC	CRC
RG7828	Lunsumio monotherapy + combos	heme tumors
CHU	clesitamig (DLL3 trispecific)	solid tumors
CHU	codrituzumab	HCC
CHU	MINT91	solid tumors
CHU	anti-CTLA-4 switch antibody	solid tumors
CHU	pan-KRAS inhibitor (AUBE00)	solid tumors
RG6421	TMEM16A potentiator	Muco-obstructive respiratory disease
RG6498	-	IBD
RG6981[4]	Il33:TSLP	COPD/Asthma
RG7828	Lunsumio	SLE
CHU	anti-C1s recycling antibody	immunology
RG6622	-	coronary artery disease
RG6631	afimkibart (anti-TL1A)	MASH
RG6035	Brainshuttle™ CD20	multiple sclerosis
RG6434	NLRP3i	neurodegenerative disorders
RG6627[5]	APOE ASO	Alzheimer's disease
RG6662	HTT miRNA GT	Huntington's Disease
RG6209	VEGF x IL6 DutaFab	DME
RG6327	-	geographic atrophy
RG6441	-	DR
RG6697	-	DME
RG6858	-	retinal diseases
RG6913	-	DME
RG6006	zosurabalpin	bacterial infections
CHU	REVN24	acute diseases

Phase II (12 NMEs + 6 AIs)

RG6114	Itovebi + enzalutamide	CRPC
RG6114	Itovebi + ribociclib + fulvestrant 1L HR+ Chr8P-loss PIK3CA-wt mBC	
RG6180	autogene cevumeran	solid tumors
RG6596	HER2 TKI (ZN-1041)	HER2+ mBC
RG6797	dirloctocogene samoparvovec (SPK-8011QQ)	hemophilia A
RG6631	afimkibart (anti-TL1A)	atopic dermatitis
RG6631	afimkibart (anti-TL1A)	rheumatoid arthritis
RG6730[6]	p40xTL1A bispecific antibody	UC
RG7828	Lunsumio	SLE
CHU	anti-HLA-DQ2.5 x gluten peptides antibody	celiac disease
RG6237	emugrobart (GYM 329)	obesity
RG6418	selnoflast (NLRP3i)	coronary artery disease
RG6652	GLP-1 RA (CT-996)	obesity/ T2D glycemic control
RG6849[7]	petrelintide	obesity +/- T2D
RG6182	MAGL inhibitor	multiple sclerosis
RG6289	nivegacetor (γ-secretase modulator)	Alzheimer's
RG6356[8]	Elevidys	DMD, <4 years
RG6501	OpRegen	geographic atrophy

RG-No - Roche/Genentech; CHU - Chugai managed; [1]MediLink managed; [2]Innovent managed; [3]In collaboration with Hansoh Pharma, RG Nr TBD; [4]Qyuns Therapeutics managed; [5]In collaboration with IONIS; [6]In collaboration with Pfizer; [7]Zealand Pharma managed; [8]US rights with Sarepta; *also developed in neurology

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

Roche Group development pipeline

Phase III (14 NMEs + 21 AIs)

RG3502	Kadcyla + T	HER-2+ eBC high-risk
RG6013	Hemlibra	Type 3 VWD
RG6026	Columvi + Polivy + R-CHP	1L DLBCL
	Columvi	r/r MCL
RG6107	PiaSky	aHUS
RG6114	Itovebi + fulvestrant	post CDKi HR+ PIK3CA-mut. BC
	Itovebi + Phesgo	1L HER2+ PIK3CA-mut. mBC
	Itovebi + CDK4/6i + letrozole	1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG6160	cevostamab	r/r MM
RG6171	giredestrant + Phesgo	1L ER+/HER2+ BC
	giredestrant + CDK4/6i	1L ET resistant ER+/HER2- BC
RG6330	divarasib	2L NSCLC
RG6330	divarasib + pembrolizumab	1L NSCLC
RG6512	zemocimig (NXT007)	hemophilia A
RGxxxx+	bexobrutideg	r/r CLL
RG7828	Lunsumio + lenalidomide	2L+ FL
RG7853	Alecensa	LA stage III NSCLC
RG6149	astegolimab	COPD
RG6299[2]	sefaxersen (ASO factor B)	IgA nephropathy
RG6631	afimkibart (anti-TL1A)	ulcerative colitis
	afimkibart (anti-TL1A)	Crohn's disease
RG6615[1]	zilebesiran	hypertension
RG6640	enicepatide (CT-388)	obesity +/- T2D
RG6881	pegozafermin	MASH with fibrosis stage F2 or F3
	pegozafermin	MASH with fibrosis stage F4
RG6102	trontinemab	early Alzheimer's
RG6168	Enspryng	MOGAD
	Enspryng	autoimmune encephalitis
RG6356[3]	Elevidys	DMD, 8 years and older
RG7845	fenebrutinib	RMS
	fenebrutinib	PPMS
RG7935	prasinezumab	Parkinson´s
RG6179	vamikibart	UME
RG6321	Susvimo	wAMD, 36-week
RG7716	Vabysmo	CNV

Registration US & EU (2 NMEs + 7 AI)

RG6171	giredestrant + everolimus†	post-CDK4/6 ER+/HER2- BC
	giredestrant†	ER+ BC adj
RG7446	Tecentriq	stage III dMMR/MSI-H colon cancer
RG7828	Lunsumio + Polivy†	2L+ DLBCL
RG7159	Gazyva	systemic lupus erythematosus
	Gazyva	childhood onset idiopathic nephrotic syndrome*
	Gazyva	membranous nephropathy
RG6168	Enspryng†	TED
RG6321	Susvimo††	wAMD

†Filed in US
††Filed in EU, approved in US

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

+RGxxxx number to be confirmed; [1]Alnylam Pharmaceuticals managed; [2]In collaboration with IONIS; [3]US rights with Sarepta; T:Tecentriq, *also known as pediatric nephrotic syndrome (PNS)

Expected regulatory submissions*

New Molecular Entities: Lead and additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
*Filed in US; +RGxxxx number to be confirmed; [1]In collaboration with IONIS ; [2]In collaboration with Pfizer; [3]Alnylam Pharmaceuticals managed; [4]Zealand Pharma managed

2026

RG6171	**giredestrant ✓*** ER+ BC adj
RG6330	**divarasib** 2L NSCLC
RG7845	**fenebrutinib** RMS &PPMS
RG6179	**vamikibart** UME

2027

RG6171	**giredestrant + Phesgo** 1L ER+/HER2+ BC
RG6171	**giredestrant + CDK4/6i** 1L ET resistant ER+/HER2- BC
RG6299[1]	**sefaxersen (ASO factor B)** IgA nephropathy
RG6631	**afimkibart (anti-TL1A)** ulcerative colitis
RG6321	**Susvimo** wAMD, 36-week refill
RG6321	**Susvimo** DME (EU)

2028

RG6512	**zemocimig (NXT007)** Hemophilia A
RG6102	**trontinemab** early Alzheimer's
RG6640	**enicepatide (CT-388)** obesity +/- T2D
RG6881	**pegozafermin** MASH with fibrosis stage F2 or F3
RG6881	**pegozafermin** MASH with fibrosis stage F4

2029 and beyond

RG6160	**cevostamab** 2-4L r/r MM
RG6180	**autogene cevumeran** solid tumors
RG6330	**divarasib + pembrolizumab** 1L NSCLC
RG6596	**HER2 TKI (ZN-1041)** 2L+ HER2+ BC
RG6797	**dirloctocogene samoparvovec** Hemophilia A
RGxxxx+	**bexobrutideg** r/r CLL
RG6631	**afimkibart (anti-TL1A)** Crohn's disease
RG6631	**afimkibart (anti-TL1A)** atopic dermatitis
RG6631	**afimkibart (anti-TL1A)** rheumatoid arthritis
CHU	**anti-HLA-DQ2.5 x gluten peptides antibody** celiac disease
RG6730[2]	**p40xTL1A bispecific antibody** UC
RG6182	**MAGL inhibitor** multiple sclerosis
RG6356	**Elevidys** DMD, <4 years
RG6356	**Elevidys** DMD, 8 yrs and older
RG6289	**nivegacetor** (γ-secretase modulator) Alzheimer's
RG7935	**prasinezumab** Parkinson's
RG6501	**OpRegen** geographic atrophy
RG6237	**emugrobart (GYM 329)** obesity
RG6418	**selnoflast (NLRP3i)** coronary artery disease
RG6615[3]	**zilebesiran** hypertension
RG6652	**GLP-1 RA (CT-996)** obesity/ T2D glycemic control
RG6849[4]	**petrelintide** obesity +/- T2D

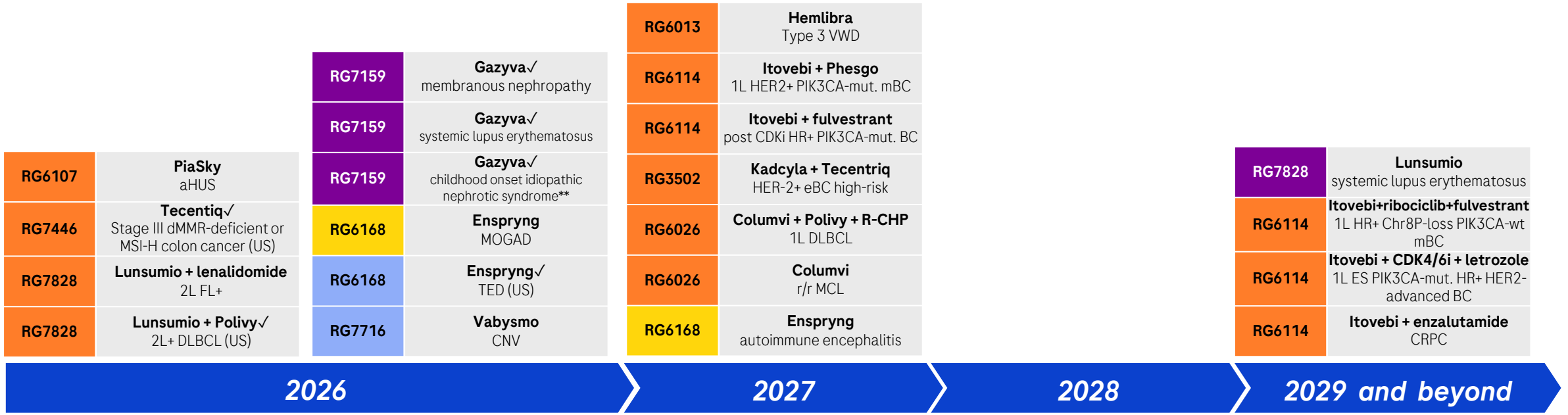
Status as of July 23, 2026

Expected regulatory submissions*

Marketed products: Additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
**also known as pediatric nephrotic syndrome (PNS)



Status as of July 23, 2026

Major pending approvals 2026

US

RG6171	**giredestrant + everolimus** post-CDK4/6 ER+/HER2- BC Filed December 2025
RG6171	**giredestrant** ER+ BC adj Filed March 2026
RG7446	**Tecentriq** stage III dMMR/MSI-H colon cancer Filed April 2026
RG7828	**Lunsumio + Polivy** 2L+ DLBCL Filed April 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed March 2026
RG7159	**Gazyva** systemic lupus erythematosus Filed February 2026
RG7159	**Gazyva** membranous nephropathy Filed June 2026
RG6168	**Enspryng** TED Filed April 2026

EU

RG7446	**Tecentriq** stage III dMMR/MSI-H colon cancer Filed July 2026
RG6321	**Susvimo** wAMD Filed July 2025
RG7159	**Gazyva** systemic lupus erythematosus Filed Feb 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed May 2026
RG7159	**Gazyva** membranous nephropathy Filed June 2026

China

RG6171	**giredestrant** ER+ BC adj Filed April 2026
RG7828	**Lunsumio + Polivy** r/r large B-cell lymphoma Filed July 2026
RG7159	**Gazyva** lupus nephritis Filed March 2026
RG7159	**Gazyva** systemic lupus erythematosus Filed March 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed April 2026
RG7159	**Gazyva** membranous nephropathy Filed May 2026
RG6168	**Enspryng** TED Filed May 2026

Japan-Chugai

RG7446	**Tecentriq** ctDNA+ high-risk MIBC Filed January 2026
RG6171	**giredestrant** post-CDK4/6 ER+/HER2- BC Filed May 2026
RG6171	**giredestrant** ER+ BC adj Filed May 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed May 2026
RG6321	**Susvimo** wAMD, DME Filed June 2026
RG7446	**Tecentriq** LA esophageal cancer maintenance Filed June 2026

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*also known as pediatric nephrotic syndrome (PNS)

Status as of July 23, 2026

Major granted approvals 2026

US		EU		China		Japan-Chugai	
RG7446	**Tecentriq** ctDNA+ high-risk MIBC May 2026			**RG7596**	**Polivy + chemo** r/r DLBCL March 2026	**RG7828**	**Lunsumio + Polivy** r/r large B-cell lymphoma March 2026
						RG7853	**Alecensa** ALK+ solid tumors May 2026
						RG435	**Avastin** neurofibromatosis Type 2 June 2026

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

Doing now what patients need next